

ENGLISH FOR CARDIOLOGISTS

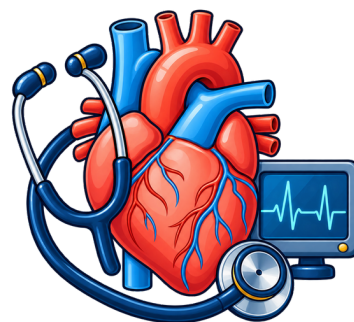
Cardiology English

Explain the finding. Clarify the decision.

Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.

No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.



Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For cardiologists, cardiology fellows, and physicians coordinating cardiovascular care.

2026 edition

Eight scenarios. Eight skills.

Eight cardiovascular encounters practice urgency, rhythm evidence, imaging explanations, stroke and bleeding tradeoffs, procedure discussions, care plans, risk numbers, and rehabilitation handoffs.

01 Giving an urgent cardiovascular handoff 4

Concern first, uncertainty intact | Communicate a time-critical concern without assigning an unconfirmed diagnosis or delaying emergency care.

02 Explaining ambulatory rhythm monitoring 13

A symptom marker is not a diagnosis | Explain what a rhythm recording can show and obtain a usable symptom account without promising detection.

03 Explaining an echocardiogram 22

A percentage is not the amount of heart left | Translate an imaging measurement into plain English without reducing the whole assessment to one number.

04 Discussing atrial-fibrillation treatment goals 31

The rhythm question and the stroke question | Separate symptom control from stroke prevention while explaining a benefit-and-bleeding discussion.

05 Discussing coronary angiography consent 40

A diagnostic test is not automatic treatment | Explain a proposed procedure and clarify what has and has not been agreed.

06 Checking a heart-failure home plan 49

Teach-back checks the explanation | Check understanding of an individualized plan without treating a printed sheet as proof of comprehension.

07 Explaining absolute and relative risk 58

Twenty-five percent of what? | Translate a relative change into comparable absolute frequencies without turning group data into a personal guarantee.

08 Coordinating rehabilitation and return to activity 67

Referred is not yet enrolled | Explain a rehabilitation referral and negotiate practical follow-up without giving an unsupplied exercise prescription.

Communication field notes	3
Three more workplace conversations	76
Answers and explanations	85
Fast-access phrase pages	104
Vocabulary index	106
Sources and publication notes	114

A precise finding is not the whole decision.

Cardiology moves between symptoms, electrical recordings, imaging, and probabilities. Clear language identifies the evidence source, explains what a number measures, and connects the next action to a responsible professional.

Lead urgent handoffs with the concern

Give the current concern and action already taken before background detail. Preserve unknowns and confirm acceptance without delaying the actual emergency response.

"The emergency pathway is active; I am confirming the receiving team."

Name the measurement

A percentage, rhythm label, or imaging estimate answers a particular question. Do not let patients mistake a measurement for a percentage of life remaining or a complete diagnosis.

"Ejection fraction describes a pumping measurement, not how much of your heart is alive."

Make comparisons comparable

Risk numbers need the same outcome, denominator, and time period. Explain absolute change as well as relative change using the supplied figures.

"That is one fewer event per hundred over the stated period, not a personal guarantee."

Keep the plan connected

A referral, device transmission, or discharge summary is not proof that follow-up occurred. Name the receiving clinician, outstanding question, and communication route.

"The rehabilitation referral is sent; the first appointment is not yet confirmed."

PROFESSIONAL BOUNDARIES

All people, records, figures, and encounters are fictional. This is professional English practice, not medical advice, clinical training, a treatment protocol, or certification. Use current evidence, authorized supervision where required, local law, and your actual clinical and emergency procedures. Do not use an exercise to decide that symptoms can safely wait, select treatment, or calculate a dose. Communication about privacy, consent, and professional duties depends on jurisdiction and setting. Use qualified language support and protect real patient information.

Concern first, uncertainty intact

A Read the situation

Communicate a time-critical concern without assigning an unconfirmed diagnosis or delaying emergency care.

Dr Vale hands over Chris, who reports chest pressure with breathlessness. The emergency response is already active. The cause, initial test results, and current measurements are not supplied in this exercise. Dr Chen is accepting the clinical handoff. The task is concise communication of reported symptoms, known actions, and unresolved information, not deciding whether chest discomfort is safe to wait.

PEOPLE / Dr Vale: Referring physician | Dr Chen: Receiving cardiologist

URGENT DOES NOT MEAN SPECULATIVE

Use a clear opening and closed confirmation. Do not increase certainty to sound decisive, and do not postpone an active emergency response until a report sounds polished.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What symptoms are reported?

- | | |
|---|---------------------------------|
| A A normal examination | B No current concern |
| C Chest pressure with breathlessness | D A confirmed infarction |

2. What is already active?

- | | |
|---|---------------------------------|
| A A routine appointment next month | B A completed discharge |
| C A prescribed medicine from this exercise | D The emergency response |

3. Who accepts the handoff?

- | | |
|--|---------------------------|
| A An unnamed waiting-list clerk | B Nobody |
| C Dr Chen | D Chris's employer |

[Answers and explanations: page 85.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

acute coronary syndrome

Group of urgent conditions involving reduced coronary blood flow.

IN USE evaluate suspected acute coronary syndrome

myocardial ischemia

Inadequate blood supply to heart muscle relative to its needs.

IN USE assess myocardial ischemia

myocardial infarction

Heart-muscle injury and death caused by ischemia under diagnostic criteria.

IN USE confirm myocardial infarction

retrosternal

Located behind the breastbone.

IN USE describe retrosternal discomfort

exertional symptom

Symptom associated with physical effort.

IN USE clarify exertional symptoms

rest symptom

Symptom occurring without physical exertion.

IN USE report symptoms at rest

diaphoresis

Marked sweating, described in clinical context.

IN USE report associated diaphoresis

dyspnea

Subjective sensation of difficult or uncomfortable breathing.

IN USE describe reported dyspnea

troponin

Cardiac-injury biomarker interpreted with timing and clinical context.

IN USE interpret a troponin result

serial testing

Repeated testing over a defined clinical sequence.

IN USE explain serial testing

hemodynamic status

State of circulation assessed through relevant clinical information.

IN USE communicate hemodynamic status

initial assessment

First clinical evaluation in the relevant encounter.

IN USE report the initial assessment

Precision check

Chest pressure and breathlessness are reported symptoms, not a confirmed infarction. Missing measurements must remain missing; the exercise cannot classify urgency or establish a diagnosis.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

prehospital report

Clinical information collected before arrival at the receiving hospital.

IN USE relay the prehospital report

emergency activation

Initiation of the relevant emergency-response process.

IN USE confirm emergency activation

symptom trajectory

Pattern of symptom change over time.

IN USE describe the symptom trajectory

receiving team

Professionals accepting the next stage of care.

IN USE identify the receiving team

time-critical concern

Concern requiring prompt action within the relevant clinical process.

IN USE communicate a time-critical concern

closed-loop escalation

Urgent communication that includes confirmation of receipt and responsibility.

IN USE complete closed-loop escalation

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Emergency activation and information transfer happen together. Do not wait for every detail or a language correction before following the real emergency process.

Phrases you can actually use

C Notice the language / 1 of 2

LEAD WITH THE CONCERN

"I am handing over chest pressure with breathlessness."

AVOID INVENTION

"I do not have a verified measurement to report."

REQUEST ACCEPTANCE

"Please confirm that your team is receiving the patient and report."

CORRECT IMMEDIATELY

"That detail needs correction before it is repeated."

STATE THE ACTION

"The emergency response is already active."

SEPARATE DIAGNOSIS

"The cause is not established in this handoff."

PRESERVE TIMING

"I will distinguish reported onset from the time of our assessment."

CLOSE THE LOOP

"I have your acknowledgment and the receiving contact."

Reports

Attributes symptoms to the patient rather than presenting an unverified finding as measured.

Already active

Identifies an emergency action that must not wait for this conversation to finish.

Confirm receiving

Requests acceptance instead of assuming that a sent message completed the transfer.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening is strongest?

- A** Chest pressure with breathlessness; the emergency response is active.
- B** A small issue may need discussion eventually.
- C** A confirmed infarction with invented results.
- D** No concern because no results are shown.

2. Which result must not be invented?

- A** The active emergency response
- B** A normal troponin
- C** The reported chest pressure
- D** The accepted handoff

3. What completes the communication loop?

- A** An unanswered voicemail alone
- B** A guessed diagnosis
- C** A promise that nothing can change
- D** Confirmation of the receiving team and responsibility

4. What should not delay emergency care?

- A** The receiving clinician's assessment
- B** Necessary clinical action
- C** Completing every detail of this language exercise
- D** The actual local emergency pathway

[Answers and explanations: page 85.](#)

Concern first, uncertainty intact

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

receiving team / dyspnea / closed-loop escalation / Myocardial infarction / Emergency activation / symptom trajectory / prehospital report / time-critical concern / troponin / hemodynamic status

- 01 **Dr Vale:** I am handing over Chris, who reports chest pressure with breathlessness. The emergency response is already active.
- 02 **Dr Chen:** I am receiving the report. Keep this ¹ _____ with the clinical team while we clarify the verified information.
- 03 **Dr Vale:** I will separate what Chris reports from what our team has actually observed or measured.
- 04 **Dr Chen:** Good. Report ² _____ as a symptom, and do not convert it into a diagnosis without the evidence.
- 05 **Dr Vale:** I do not yet have verified current measurements to pass on. I will update you as they become available.
- 06 **Dr Chen:** Then do not describe the ³ _____ as normal. Missing information must stay visible while the real assessment continues.
- 07 **Dr Vale:** I also need to distinguish the patient's reported onset from the time someone first contacted the service.
- 08 **Dr Chen:** Yes. The ⁴ _____ should retain its source and timing rather than be blended with later observations.
- 09 **Dr Vale:** I have not labeled this a confirmed infarction. The symptoms justify concern, but the cause is unestablished here.
- 10 **Dr Chen:** Correct. ⁵ _____ is not a substitute label for every chest-pressure report.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Dr Vale:** Nor should an absent test result be treated as a negative test that rules out the concern.
- 12 **Dr Chen:** Exactly. We are still awaiting the ⁶ _____ result. Please include the sampling time when you pass it on.
- 13 **Dr Vale:** I will send any verified update through the active pathway rather than leave a separate routine message.
- 14 **Dr Chen:** Please do. The ⁷ _____ needs to know which facts are new and who is responsible for them.
- 15 **Dr Vale:** Shall I wait until every field is complete before confirming transfer of the report?
- 16 **Dr Chen:** No. ⁸ _____ is already underway; do not interrupt necessary care to perfect the wording.
- 17 **Dr Vale:** I will identify the unfinished information explicitly and confirm the contact for further clinical updates.
- 18 **Dr Chen:** That supports ⁹ _____. I acknowledge the concern and am accepting this handoff through our clinical process.
- 19 **Dr Vale:** Understood. Reported symptoms, active response, unresolved cause, and no invented measurements or results.
- 20 **Dr Chen:** Correct. Continue communicating the ¹⁰ _____ as verified information becomes available, alongside the real clinical response.

ANSWERS / EXPLANATIONS: PAGE 86

1. time-critical concern | 2. dyspnea | 3. hemodynamic status | 4. prehospital report | 5. Myocardial infarction | 6. troponin | 7. receiving team | 8. Emergency activation | 9. closed-loop escalation | 10. symptom trajectory

A corrected onset time

F Say it

During an active emergency handoff, the patient clarifies that symptoms began at 09:10, not 09:40. The receiving clinician acknowledges the correction. No cause is established.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unestablished / active / 09:10 / 09:40

Referrer: "The corrected reported onset is ¹ _____."

Receiver: "The earlier time of ² _____ is superseded."

Referrer: "The emergency response remains ³ _____."

Receiver: "The cause remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. 09:10 | 2. 09:40 | 3. active | 4. unestablished

Rehearse with a fixed script

1. Read the corrected urgent handoff. Lead with the symptom report and emergency action.
2. Swap roles. Repeat the acceptance and missing-information statements without inserting measurements or a diagnosis.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A symptom marker is not a diagnosis

A Read the situation

Explain what a rhythm recording can show and obtain a usable symptom account without promising detection.

Ellis describes intermittent palpitations. Dr Mora proposes an ambulatory recording to compare symptoms with heart rhythm. No arrhythmia has been established in this case. Ellis will receive device-specific instructions from the service. A symptom diary and event marker may help interpretation; they do not themselves diagnose the sensation or guarantee that every episode will be captured.

PEOPLE / Ellis: Patient | Dr Mora: Cardiologist

EXPLAIN THE PURPOSE BEFORE THE EQUIPMENT

A patient needs to know what question the monitor addresses, not just how long it is worn. Keep emergency instructions separate from routine recording instructions.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is the proposed purpose?

- A** Prove an arrhythmia already exists
- B** Replace all urgent assessment
- C** Compare symptoms with recorded rhythm
- D** Guarantee every episode is captured

2. What is not established?

- A** An arrhythmia
- B** Ellis's symptom report
- C** A proposed recording
- D** The need for instructions

3. Who provides equipment instructions?

- A** The service using the actual device
- B** A guessed universal script
- C** Another patient
- D** The learner inventing settings

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

palpitations

Awareness of the heartbeat as unusual, forceful, fast, or irregular.

IN USE describe palpitations

electrocardiogram

Recording of the heart's electrical activity.

IN USE interpret an electrocardiogram

arrhythmia

Abnormality of heart rhythm.

IN USE investigate a suspected arrhythmia

sinus rhythm

Rhythm originating from the heart's usual sinus-node pacemaker.

IN USE identify sinus rhythm

ectopic beat

Beat arising outside the usual initiating site.

IN USE describe an ectopic beat

ambulatory monitor

Device recording heart rhythm during everyday activity.

IN USE fit an ambulatory monitor

event marker

Device feature marking a time for later review.

IN USE use the event marker

symptom diary

Record of symptoms, times, and relevant activity.

IN USE keep a symptom diary

recording period

Time during which the device collects data.

IN USE confirm the recording period

symptom-rhythm correlation

Relationship between reported symptoms and rhythm at the relevant time.

IN USE assess symptom-rhythm correlation

paroxysmal

Occurring in episodes with onset and offset.

IN USE describe paroxysmal symptoms

bradycardia

Heart rate slower than expected in the relevant context.

IN USE assess bradycardia

Precision check

Palpitations are a symptom; arrhythmia is a rhythm finding. A marked event helps locate information but does not itself diagnose it.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

tachycardia

Heart rate faster than expected in the relevant context.

IN USE assess tachycardia

conduction

Transmission of electrical impulses through the heart.

IN USE evaluate cardiac conduction

electrode lead

Electrical recording connection used in acquiring a trace.

IN USE check an electrode lead

signal artifact

Distortion in a recording not representing the true underlying signal.

IN USE identify signal artifact

signal quality

Adequacy of the recorded signal for interpretation.

IN USE assess signal quality

monitor return

Process of returning equipment or transmitting its recorded data.

IN USE confirm monitor return

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A routine monitor is not a substitute for the service's urgent-care instructions. Do not advise waiting for a recording review when new concerning symptoms require actual assessment.

Phrases you can actually use

C Notice the language / 1 of 2

EXPLAIN THE PURPOSE

"We want to compare what you feel with the rhythm at that time."

LIMIT THE PROMISE

"The recording may not capture every episode."

CLARIFY THE MARKER

"The marker identifies a time for review, not a diagnosis."

SEPARATE URGENT CARE

"Follow the urgent-care instructions rather than waiting for the monitor report."

SEPARATE SYMPTOM AND FINDING

"Palpitations do not name a specific rhythm by themselves."

MAKE THE DIARY USEFUL

"Include the time, what you felt, and what you were doing."

USE THE ACTUAL INSTRUCTIONS

"The service will explain this device's controls and restrictions."

CLOSE THE PROCESS

"Let us confirm how the recording reaches the reviewing team."

At that time

Connects the reported sensation with the corresponding recording interval.

May not capture

Expresses a limitation without implying that the recording is useless.

Rather than waiting

Separates an urgent-care route from routine data collection.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

MAKE SPACE

"We can pause before moving to the next point."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

EXPLAIN A TERM

"In everyday language, that means the following."

CHECK YOUR EXPLANATION

"Let me explain that another way."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which distinction is correct?

- A** Palpitations are a symptom; an arrhythmia is a rhythm abnormality.
- B** Every palpitation proves an arrhythmia.
- C** A diary is a diagnosis.
- D** A marker guarantees a dangerous rhythm.

2. What makes a diary entry useful?

- A** Time, sensation, and activity
- B** Only the patient's opinion of the device color
- C** A diagnosis invented at home
- D** An unrelated appointment number

3. Which promise is unsupported?

- A** A marker identifies an event time.
- B** Every episode will definitely be captured.
- C** The device has specific instructions.
- D** The recording needs interpretation.

4. What must remain separate?

- A** Urgent-care instructions and routine monitor review
- B** Symptom time and the corresponding trace
- C** A device and its actual instructions
- D** The report and its reviewing team

Answers and explanations: [page 87](#).

A symptom marker is not a diagnosis

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Signal artifact / ambulatory monitor / symptom diary / symptom-rhythm correlation / palpitations / monitor return / recording period / arrhythmia / event marker / signal quality

- 01 **Ellis:** My heart sometimes feels as though it skips and then thumps. Is that enough to name the rhythm problem?
- 02 **Dr Mora:** Those are ¹ _____, a symptom description. We need appropriate evidence before naming a particular rhythm.
- 03 **Ellis:** The episodes come and go. They never seem to happen at the exact moment someone checks me.
- 04 **Dr Mora:** An ² _____ can record during ordinary activity, helping us look for a relationship with the episodes.
- 05 **Ellis:** Does agreeing to the monitor mean you have already diagnosed an arrhythmia from my account?
- 06 **Dr Mora:** No. An ³ _____ has not been established in this case. The recording is part of investigating the question.
- 07 **Ellis:** What should I record when I notice the feeling, besides pressing whatever button the device has?
- 08 **Dr Mora:** The service will explain the device. A ⁴ _____ should include the time, what you feel, and what you are doing.
- 09 **Ellis:** And pressing a marker does not mean the device has instantly told me what caused the sensation?
- 10 **Dr Mora:** Correct. The ⁵ _____ identifies a time to review; it is not itself a diagnosis or a promise of real-time assessment.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Ellis:** I am worried I will wear it and nothing will happen during the recording period.
- 12 **Dr Mora:** That is a possible limitation. We are seeking ⁶ _____, and the available recording may not capture every intermittent event.
- 13 **Ellis:** If the trace looks strange while I move, will every unusual line be treated as a heart problem?
- 14 **Dr Mora:** No. ⁷ _____ can affect recordings, so quality and context matter to the interpretation.
- 15 **Ellis:** Then I should follow the actual device instructions rather than borrow instructions from a friend's different monitor.
- 16 **Dr Mora:** Yes. Those instructions help protect ⁸ _____ and explain the controls, restrictions, and reporting process for this device.
- 17 **Ellis:** If I develop a concerning new symptom, I should not assume pressing the marker replaces asking for help.
- 18 **Dr Mora:** Exactly. Follow the actual urgent-care instructions; the ⁹ _____ is not permission to delay needed clinical assessment.
- 19 **Ellis:** I also want to know how you receive the recording and when the team explains its findings.
- 20 **Dr Mora:** We will confirm the ¹⁰ _____ process and the service's review arrangements, without promising a result the data may not establish.

ANSWERS / EXPLANATIONS: PAGE 88

1. palpitations | 2. ambulatory monitor | 3. arrhythmia | 4. symptom diary | 5. event marker | 6. symptom-rhythm correlation | 7. Signal artifact | 8. signal quality | 9. recording period | 10. monitor return

A diary time needs clarification

F Say it

A diary says an episode occurred at 7, without morning or evening. The reviewing team asks Ellis to clarify before matching it to the trace. No rhythm conclusion is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

clarify / supplied / evening / trace

Reviewer: "The entry gives seven but not morning or ¹ _____."

Ellis: "I need to ² _____ that detail."

Reviewer: "Then we can match it to the ³ _____."

Ellis: "No rhythm conclusion has been ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. evening | 2. clarify | 3. trace | 4. supplied

Rehearse with a fixed script

1. Read the corrected monitoring discussion. Contrast symptom, marker, and interpreted rhythm.
2. Swap roles. Repeat the device-instruction and urgent-care distinctions without inventing equipment settings.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A percentage is not the amount of heart left

A Read the situation

Translate an imaging measurement into plain English without reducing the whole assessment to one number.

Ren reads an echocardiogram report containing a left-ventricular ejection-fraction measurement. No numerical value or heart-failure diagnosis is supplied. Ren thinks the percentage describes how much heart muscle is still alive. Dr Kim explains what the measurement represents and why symptoms, structure, filling, valves, and other findings also matter.

PEOPLE / Ren: Patient | Dr Kim: Cardiologist

ASK WHAT THE NUMBER MEANS TO THE LISTENER

A percentage can sound intuitive while being misunderstood. Establish the patient's interpretation before adding ranges or categories that may create a second misunderstanding.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Ren misunderstand?

- | | |
|--------------------------------------|--|
| A The appointment date | B A prescribed dose |
| C A confirmed valve procedure | D The meaning of the ejection-fraction percentage |

2. What is not supplied?

- | | |
|-----------------------------------|---|
| A An echocardiogram report | B Ren's question |
| C A clinician explanation | D A numerical value or heart-failure diagnosis |

3. What else belongs in interpretation?

- | | |
|--|-------------------------------|
| A The report's font size | B A guessed comparison |
| C Symptoms and other cardiac findings | D Only the percentage |

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

echocardiogram

Ultrasound examination of heart structure and function.

IN USE explain an echocardiogram

ejection fraction

Proportion of ventricular blood volume expelled with a contraction.

IN USE explain left-ventricular ejection fraction

systole

Phase of the cardiac cycle involving ventricular contraction and ejection.

IN USE describe systolic function

diastole

Phase involving ventricular relaxation and filling.

IN USE describe diastolic filling

left ventricle

Heart chamber pumping blood into the systemic circulation.

IN USE assess the left ventricle

right ventricle

Heart chamber pumping blood toward the lungs.

IN USE assess the right ventricle

stroke volume

Amount of blood ejected by a ventricle in one beat.

IN USE distinguish stroke volume

preserved ejection fraction

Ejection fraction within the relevant preserved category despite possible other dysfunction.

IN USE discuss preserved ejection fraction

reduced ejection fraction

Ejection fraction below the relevant category threshold in context.

IN USE explain reduced ejection fraction

valvular regurgitation

Backward flow through a valve that does not close adequately.

IN USE assess valvular regurgitation

valvular stenosis

Narrowing that restricts flow through a heart valve.

IN USE explain valvular stenosis

wall motion

Movement of heart-muscle segments during the cardiac cycle.

IN USE assess regional wall motion

Precision check

Ejection fraction is a pumping proportion, not a percentage of living heart tissue or life expectancy. It is not the whole assessment of cardiac function.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

diastolic function

Ventricular relaxation and filling characteristics.

IN USE assess diastolic function

structural heart disease

Abnormality of heart anatomy or associated structures.

IN USE investigate structural heart disease

ventricular volume

Amount of blood within a ventricle at a specified phase.

IN USE measure ventricular volume

pressure estimate

Indirect approximation of pressure from available measurements.

IN USE qualify a pressure estimate

image quality

Adequacy of images for the intended interpretation.

IN USE describe image quality

comparison study

Earlier examination used to evaluate change.

IN USE review a comparison study

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Preserved ejection fraction does not, by itself, exclude heart failure. Interpret the actual measurement with the clinical context and other findings; this case supplies no diagnostic category.

Phrases you can actually use

C Notice the language / 1 of 2

03

CHECK THE INTERPRETATION

"What did that percentage suggest to you?"

CORRECT GENTLY

"It is not the percentage of your heart that is still alive."

BROADEN THE PICTURE

"Filling, valves, symptoms, and other findings also matter."

AVOID A SINGLE-NUMBER DIAGNOSIS

"We cannot interpret the whole situation from one number alone."

DEFINE THE MEASUREMENT

"It describes the proportion of blood pushed out with a contraction."

NAME THE CHAMBER

"This report refers to the left ventricle."

QUALIFY THE IMAGE

"The report should state any important image-quality limitation."

SEPARATE A COMPARISON

"A change needs a comparable earlier study and clinical context."

Proportion versus amount

Ejection fraction is a proportion, whereas stroke volume is an amount per beat.

Not the percentage of

Corrects exactly what the number does not measure.

By itself

Limits the conclusion supported by one measurement without dismissing its usefulness.

Phrases you can actually use

C Notice the language / 2 of 2

03

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. What does ejection fraction measure?

- A** Life expectancy in percent
- B** The percentage of arteries without narrowing
- C** A proportion of ventricular blood expelled with contraction
- D** The percentage of living heart tissue

2. Which statement is accurate?

- A** Preserved ejection fraction alone does not exclude heart failure.
- B** A preserved value proves every cardiac function is normal.
- C** The percentage is a prognosis timetable.
- D** A low value identifies every cause.

3. Which distinction is useful?

- A** Both mean the number of heartbeats.
- B** Both measure the patient's age.
- C** Neither refers to pumping.
- D** Stroke volume is an amount; ejection fraction is a proportion.

4. Which conclusion exceeds this case?

- A** The terminology needs explanation.
- B** No numerical value is supplied.
- C** Ren has a specific heart-failure category.
- D** Ren has a report.

Answers and explanations: [page 89](#).

A percentage is not the amount of heart left

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

valvular regurgitation / ventricular volume / Ejection fraction / Preserved ejection fraction / Diastolic function / left ventricle / Stroke volume / image quality / comparison study / Systole

- 01 **Ren:** The report gives a percentage. I thought it meant that only that much of my heart is still alive.
- 02 **Dr Kim:** ¹ _____ does not measure how much heart tissue remains alive. Let me explain what the number represents.
- 03 **Ren:** Please do. I have been thinking of it as a percentage score for the whole heart.
- 04 **Dr Kim:** This measurement concerns the ² _____, the chamber that pumps blood toward the body.
- 05 **Ren:** What is the percentage actually a percentage of, if it is not the amount of working muscle?
- 06 **Dr Kim:** It relates the blood expelled to the ³ _____ before that contraction. It describes a pumping proportion.
- 07 **Ren:** Then a report using an amount of blood per beat would be describing something related but different?
- 08 **Dr Kim:** Yes. ⁴ _____ is an amount per beat, while ejection fraction expresses a proportion.
- 09 **Ren:** I also saw systolic and diastolic in the report. Are those two different parts of the heartbeat?
- 10 **Dr Kim:** ⁵ _____ involves contraction and ejection. Diastole involves relaxation and filling; both are relevant to function.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Ren:** So the part about filling is not just an extra technical detail that can be ignored?
- 12 **Dr Kim:** Correct. ⁶ _____ is part of the assessment, along with structure, valves, symptoms, and other findings.
- 13 **Ren:** If someone has a preserved percentage, does that prove they cannot have heart failure?
- 14 **Dr Kim:** No. ⁷ _____ alone does not exclude it. We need the complete clinical picture, not just that label.
- 15 **Ren:** I understand. The report also discusses a valve, which I assumed the percentage already covered.
- 16 **Dr Kim:** Valve findings are assessed separately. For example, ⁸ _____ describes backward flow through a valve, not the ejection fraction itself.
- 17 **Ren:** What if part of the scan was hard to see clearly? Should that limitation be explained too?
- 18 **Dr Kim:** Yes. Important ⁹ _____ limitations should be stated rather than hidden behind a precise-looking number.
- 19 **Ren:** Then I should ask how this fits the rest of the assessment, not treat it as an isolated score.
- 20 **Dr Kim:** Exactly. Any ¹⁰ _____ also needs context. We will explain the actual findings without inventing a diagnosis from one measurement.

ANSWERS / EXPLANATIONS: PAGE 90

1. Ejection fraction | 2. left ventricle | 3. ventricular volume | 4. Stroke volume | 5. Systole | 6. Diastolic function | 7. Preserved ejection fraction | 8. valvular regurgitation | 9. image quality | 10. comparison study

A comparison needs context

F Say it

A report cites an earlier scan, but the cardiologist has not yet reviewed whether the studies are comparable. No meaningful change has been established.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

context / comparable / established / scan

Patient: "The report refers to an earlier ¹ _____."

Clinician: "I still need to review whether they are ² _____."

Patient: "A meaningful change is not ³ _____."

Clinician: "We will interpret the findings in ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. scan | 2. comparable | 3. established | 4. context

Rehearse with a fixed script

1. Read the corrected echo explanation. Contrast proportion, amount, and living tissue.
2. Swap roles. Repeat the preserved-ejection-fraction explanation without assigning Ren a diagnosis.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The rhythm question and the stroke question

04

A Read the situation

Separate symptom control from stroke prevention while explaining a benefit-and-bleeding discussion.

Avery has an established atrial-fibrillation diagnosis and asks why stroke prevention is discussed when the heartbeat feels calmer. Dr Patel will explain that symptom or rhythm management and stroke prevention address related but different questions. No risk score, medicine selection, dose, or instruction to stop treatment is supplied. Avery is worried about bleeding and wants that concern included.

PEOPLE / Avery: Patient | Dr Patel: Cardiologist

EXPLAIN WHY TWO GOALS REMAIN ON THE AGENDA

Feeling better can be mistaken for proof that every risk has disappeared. Acknowledge the improvement while explaining the separate question and taking bleeding concerns seriously.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What diagnosis is established?

- A** No rhythm diagnosis
- B** Atrial fibrillation
- C** A new stroke in this case
- D** A bleeding disorder diagnosed here

2. What does Avery ask about?

- A** A dose already supplied
- B** A confirmed procedure date
- C** A guaranteed risk-free medicine
- D** Why stroke prevention matters when symptoms feel calmer

3. What is absent from the case?

- A** A consultation
- B** A medicine selection or risk score
- C** Avery's bleeding concern
- D** The diagnosis

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

atrial fibrillation

Irregular atrial electrical activity producing an irregular heart rhythm.

IN USE explain atrial fibrillation

atrial flutter

Organized rapid atrial rhythm distinct from atrial fibrillation.

IN USE distinguish atrial flutter

rate control

Strategy aimed at controlling heart rate.

IN USE discuss rate control

rhythm control

Strategy aimed at restoring or maintaining an appropriate rhythm.

IN USE discuss rhythm control

anticoagulation

Treatment reducing blood's ability to form clots through coagulation pathways.

IN USE review anticoagulation

antiplatelet therapy

Treatment reducing platelet activity in clot formation.

IN USE distinguish antiplatelet therapy

stroke risk

Likelihood of stroke in a defined clinical context and period.

IN USE assess stroke risk

bleeding risk

Likelihood of bleeding under relevant clinical circumstances.

IN USE assess bleeding risk

thromboembolism

Obstruction caused by a clot traveling from another site.

IN USE discuss thromboembolism

cardioversion

Intervention intended to restore an appropriate heart rhythm.

IN USE explain cardioversion

catheter ablation

Procedure targeting tissue involved in an abnormal rhythm.

IN USE discuss catheter ablation

left atrial appendage

Pouch-like structure attached to the left atrium.

IN USE explain the left atrial appendage

Precision check

Feeling fewer palpitations does not, by itself, settle the stroke-prevention decision. Do not derive an instruction to start or stop a medicine from this language exercise.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

renal function

How effectively the kidneys perform their relevant functions.

IN USE review renal function

drug interaction

Effect of one medicine or substance on another's action.

IN USE check for a drug interaction

shared treatment decision

Care choice informed by evidence and the person's priorities.

IN USE reach a shared treatment decision

bleeding history

Account of previous bleeding events and related circumstances.

IN USE clarify the bleeding history

individual risk estimate

Context-dependent estimate for a person's likelihood of an outcome.

IN USE explain an individual risk estimate

anticoagulation review

Clinical reassessment of anticoagulant use and relevant risks.

IN USE arrange an anticoagulation review

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Anticoagulants and antiplatelets are not interchangeable labels. Explain their roles accurately within the individual clinical discussion rather than treating all "blood thinners" as identical.

Phrases you can actually use

C Notice the language / 1 of 2

SEPARATE THE GOALS

"Calming symptoms and preventing stroke are related but different goals."

KEEP ASSESSMENT INDIVIDUAL

"We need to assess your stroke and bleeding risks in context."

CLARIFY A COLLOQUIAL TERM

"Blood thinner can refer to medicines with different actions."

EXPLORE THE HISTORY

"Tell me about any previous bleeding and the circumstances."

ACKNOWLEDGE IMPROVEMENT

"It is useful to hear that the heartbeat feels calmer."

INVITE THE CONCERN

"What worries you most about bleeding?"

AVOID AN INSTRUCTION

"This explanation is not a direction to stop your medicine."

AGREE ON REVIEW

"Let us review the relevant information before settling the plan."

Related but different

Connects the goals without treating one as a substitute for the other.

By itself

Limits the significance of improved symptoms for a separate risk decision.

Before settling

Keeps the choice open until the required individual review is complete.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which distinction is correct?

- A** Rhythm or symptom control does not replace the stroke-prevention assessment.
- B** Calmer symptoms prove stroke risk is zero.
- C** Every medicine has the same action.
- D** A procedure automatically settles all future risk decisions.

2. Which question includes Avery's concern?

- A** What previous bleeding have you experienced, and what worries you?
- B** Why are you being difficult about risk?
- C** Can we ignore bleeding?
- D** You agree there can be no harm?

3. Which instruction cannot be supplied here?

- A** Stop anticoagulation because the heartbeat feels calmer.
- B** Discuss the goals separately.
- C** Clarify the medicine name.
- D** Review the bleeding history.

4. Why clarify blood thinner?

- A** It always identifies one exact product.
- B** It proves the dose.
- C** It means the blood becomes water.
- D** The informal phrase can hide different medicine actions.

Answers and explanations: [page 91](#).

The rhythm question and the stroke question

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Anticoagulation / Rate control / drug interaction / anticoagulation review / individual risk estimate / bleeding risk / atrial fibrillation / bleeding history / shared treatment decision / stroke risk

- 01 **Avery:** My heartbeat feels calmer now. Why are we still discussing stroke prevention as though nothing has changed?
- 02 **Dr Patel:** With ¹ _____, symptom improvement and stroke prevention are related but different parts of care.
- 03 **Avery:** I assumed that if I felt fewer episodes, the risk question would disappear with the symptoms.
- 04 **Dr Patel:** Feeling better is valuable, but ² _____ cannot be judged from how noticeable the heartbeat feels alone.
- 05 **Avery:** Please explain the terms rate control and rhythm control. They sound as though they mean exactly the same thing.
- 06 **Dr Patel:** ³ _____ concerns the heart rate; rhythm control concerns the rhythm itself. Neither phrase alone settles the prevention decision.
- 07 **Avery:** I would like the bleeding concern discussed as carefully as the benefit, not treated as a reason to stop asking.
- 08 **Dr Patel:** We should assess ⁴ _____ and your concerns alongside the possible benefit of a proposed option.
- 09 **Avery:** I have called all these medicines blood thinners, but perhaps that hides an important difference.
- 10 **Dr Patel:** Yes. ⁵ _____ and antiplatelet treatment have different actions. We should use the actual medicine name and purpose.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Avery:** I had bleeding in the past, but I am not sure which details are relevant for this discussion.
- 12 **Dr Patel:** Tell me what happened and when. Your ⁶ _____ deserves a careful account rather than an unexplained yes-or-no label.
- 13 **Avery:** Will other medicines and health conditions also affect how you judge the options for me?
- 14 **Dr Patel:** Yes. A ⁷ _____, kidney function, and other relevant factors may need review in the actual assessment.
- 15 **Avery:** Then a general chart online cannot tell me precisely which treatment I should take or at what dose.
- 16 **Dr Patel:** Correct. An ⁸ _____ needs your full history. We will review that together before discussing what the result means for you.
- 17 **Avery:** I also want to understand what matters most to me in choosing among appropriate options.
- 18 **Dr Patel:** That belongs in a ⁹ _____, with accurate information about benefits, harms, and practical demands.
- 19 **Avery:** I understand this explanation does not mean I should stop a medicine because I feel better.
- 20 **Dr Patel:** Exactly. Any change requires the appropriate ¹⁰ _____ and an individualized plan from your treating team.

ANSWERS / EXPLANATIONS: PAGE 92

1. atrial fibrillation | 2. stroke risk | 3. Rate control | 4. bleeding risk | 5. Anticoagulation | 6. bleeding history | 7. drug interaction | 8. individual risk estimate | 9. shared treatment decision | 10. anticoagulation review

Clarify which medicine is meant

F Say it

Nico says blood thinner but cannot recall the product. The clinician will verify its name and purpose before discussing a change. No stop instruction has been given.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

name / purpose / review / given

Nico: "I do not recall the product ¹ _____."

Clinician: "We will verify its name and ² _____."

Nico: "No stop instruction has been ³ _____."

Clinician: "A change needs individual ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. name | 2. purpose | 3. given | 4. review

Rehearse with a fixed script

1. Read the corrected atrial-fibrillation discussion. Contrast symptom control with stroke prevention.
2. Swap roles. Repeat the bleeding-history question and the explicit boundary against inferring a medicine change.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A diagnostic test is not automatic treatment

A Read the situation

05

Explain a proposed procedure and clarify what has and has not been agreed.

Dr Lewis discusses proposed diagnostic coronary angiography with Blair. Blair assumes a stent will definitely be inserted during the same visit. The exercise supplies no coronary anatomy, intervention decision, risk percentage, or preparation instructions. Dr Lewis must explain the diagnostic purpose, address questions, and clarify the actual consent arrangements for any possible intervention rather than treat attendance as agreement.

PEOPLE / Blair: Patient | Dr Lewis: Cardiologist

EXPLAIN BEFORE ASKING FOR AGREEMENT

A familiar procedure name may hide different expectations. Ask what the patient thinks will happen, then clarify the intended test and any separately discussed intervention.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is proposed?

- A A guaranteed normal result
- C A stent already selected

- B Diagnostic coronary angiography
- D Surgery already completed

2. What does Blair assume?

- A A result already exists
- C A stent will definitely be inserted

- B The appointment is a rehabilitation session
- D No procedure is proposed

3. What is not supplied?

- A A patient question
- C The diagnostic purpose

- B A consent discussion
- D Coronary anatomy or an intervention decision

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

coronary angiography

Imaging of coronary arteries using an appropriate angiographic technique.

IN USE explain coronary angiography

cardiac catheterization

Procedure using a catheter to investigate or treat cardiac conditions.

IN USE discuss cardiac catheterization

catheter laboratory

Specialist environment for catheter-based cardiac procedures.

IN USE attend the catheter laboratory

vascular access

Route of entry into a blood vessel for a procedure.

IN USE explain vascular access

radial access

Vessel access through the radial artery.

IN USE discuss radial access

femoral access

Vessel access through the femoral artery.

IN USE discuss femoral access

contrast medium

Substance used to improve visibility of structures on imaging.

IN USE explain contrast medium

coronary anatomy

Arrangement and features of the coronary arteries.

IN USE assess coronary anatomy

coronary narrowing

Reduction in the internal opening of a coronary artery.

IN USE evaluate coronary narrowing

percutaneous coronary intervention

Catheter-based procedure used to treat appropriate coronary artery disease.

IN USE discuss percutaneous coronary intervention

stent

Device placed to help maintain an opening in a vessel or structure.

IN USE explain a coronary stent

diagnostic procedure

Procedure intended to obtain information about a condition.

IN USE distinguish a diagnostic procedure

Precision check

Diagnostic angiography and coronary intervention are different actions. Possible same-session treatment must be explained within the actual plan and consent process; it is not automatic.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

procedural risk

Possible harm associated with a procedure.

IN USE discuss procedural risk

sedation

Medicines used to reduce awareness, anxiety, or discomfort to an intended degree.

IN USE explain the sedation plan

procedure consent

Agreement to a specified procedure after appropriate discussion.

IN USE confirm procedure consent

access-site complication

Problem occurring at the vessel-entry location.

IN USE explain access-site complications

postprocedure observation

Monitoring after a procedure under the actual care plan.

IN USE explain postprocedure observation

preparation instruction

Direction needed before a procedure for the individual plan.

IN USE verify preparation instructions

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Do not supply fasting, medicine-holding, access-site, or sedation instructions from a language exercise. Those require the actual team's individualized instructions.

Phrases you can actually use

C Notice the language / 1 of 2

CHECK EXPECTATIONS

"What have you understood will happen during the visit?"

SEPARATE TREATMENT

"A diagnostic test is not an automatic decision to insert a stent."

DISCUSS RISK ACCURATELY

"We will explain the risks relevant to the actual procedure and your situation."

VERIFY PREPARATION

"Use the instructions from the team doing your procedure."

STATE THE DIAGNOSTIC AIM

"The proposed test is intended to show the coronary arteries."

CLARIFY THE AGREEMENT

"We need to discuss exactly what you are consenting to."

INVITE ALTERNATIVES

"Please ask about reasonable alternatives and what they would establish."

KEEP THE DECISION OPEN

"No intervention decision is supplied by this conversation alone."

Intended to show

States a diagnostic purpose rather than a guaranteed finding.

Exactly what

Requests the scope of agreement instead of relying on a vague procedure label.

Possible versus planned

A possible intervention is not the same as a settled treatment plan.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which distinction is essential?

- A** Every angiogram includes a stent.
- B** Attendance is consent to any procedure.
- C** The anatomy is known before investigation.
- D** Diagnostic angiography does not automatically mean stent insertion.

2. Which instruction would be inappropriate to invent?

- A** An explanation that anatomy is not supplied
- B** A medication-holding schedule
- C** A request to clarify consent
- D** A question about expectations

3. What does informed agreement require here?

- A** A promise of zero risk
- B** A clear discussion of the specified procedure and relevant options
- C** A signature without explanation
- D** An assumption based on appointment attendance

4. Which statement preserves the facts?

- A** No intervention decision is established in this case.
- B** A stent is definitely needed.
- C** Every artery is normal.
- D** A risk percentage has been supplied.

Answers and explanations: [page 93](#).

A diagnostic test is not automatic treatment

E Conversations / turns 1-10

05

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

preparation instructions / Percutaneous coronary intervention / stent / coronary angiography / postprocedure observation / vascular access / diagnostic procedure / contrast medium / procedure consent / coronary anatomy

- 01 **Blair:** I told my family I am coming in to have a stent. Is that the right description of the appointment?
- 02 **Dr Lewis:** We are discussing ¹ _____, a diagnostic investigation. Let us clarify the difference before you make arrangements around an assumption.
- 03 **Blair:** I thought the test and the treatment were simply two names for what always happens together.
- 04 **Dr Lewis:** A ² _____ gathers information. Treatment may be discussed in relation to findings, but it is not automatic.
- 05 **Blair:** Then the purpose is to understand the arteries, rather than promise that a particular device will be inserted?
- 06 **Dr Lewis:** Yes. It helps assess ³ _____. We need to explain how any findings would affect the options before you agree to proceed.
- 07 **Blair:** Please explain the phrase PCI if it comes up. I hear abbreviations and lose track of which action is meant.
- 08 **Dr Lewis:** ⁴ _____ is a catheter-based treatment approach. We should explain the actual proposal rather than hide it behind initials.
- 09 **Blair:** What would the consent discussion need to cover if treatment during the same visit were a possibility?
- 10 **Dr Lewis:** The ⁵ _____ must be clear about the proposed actions, alternatives, relevant risks, and your questions.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

- 11 **Blair:** I do not want signing for a test to be treated as agreement to something nobody has explained.
- 12 **Dr Lewis:** That is why we clarify scope. A ⁶ _____ is not something we should describe as already decided when it has not been.
- 13 **Blair:** I have read different stories about the wrist and groin. Should I choose from those accounts myself?
- 14 **Dr Lewis:** The team will discuss the actual ⁷ _____ plan and its implications with you. Do not infer your plan from another person's experience.
- 15 **Blair:** I also want to ask about the dye and any concerns in my own medical history.
- 16 **Dr Lewis:** We should review those concerns when explaining the ⁸ _____ and the procedure's risks in your individual circumstances.
- 17 **Blair:** Should I follow preparation advice I found online while I wait for the written instructions?
- 18 **Dr Lewis:** Follow the team's ⁹ _____. If another source gives different fasting or medicine advice, please contact us to clarify it before making a change.
- 19 **Blair:** I can explain the visit more accurately now: a diagnostic discussion, with any intervention requiring clarity.
- 20 **Dr Lewis:** Good. We will also explain ¹⁰ _____ and practical arrangements, without promising an outcome or discharge time we have not established.

ANSWERS / EXPLANATIONS: PAGE 94

1. coronary angiography | 2. diagnostic procedure | 3. coronary anatomy | 4. Percutaneous coronary intervention | 5. procedure consent | 6. stent | 7. vascular access | 8. contrast medium | 9. preparation instructions | 10. postprocedure observation

A preparation sheet needs clarification

F Say it

Morgan receives two different preparation sheets. The procedural team will verify which applies. Morgan has not been told to alter medicines in this scenario.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

verify / authorized / clarified / instructions

Morgan: "The two sheets give different ¹ _____."

Clinician: "The procedural team will ² _____ which applies."

Morgan: "No medicine change is ³ _____ here."

Clinician: "The correct plan is not yet ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. instructions | 2. verify | 3. authorized | 4. clarified

Rehearse with a fixed script

1. Read the corrected procedure discussion. Contrast diagnostic information with an intervention decision.
2. Swap roles. Repeat the consent-scope question and the instruction to use the actual team's preparation advice.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Teach-back checks the explanation

A Read the situation

Check understanding of an individualized plan without treating a printed sheet as proof of comprehension.

Dr Evans reviews an individualized heart-failure plan with Lee. The actual plan includes symptom monitoring, a verified medicine list, contact routes, and follow-up. No weight threshold, fluid limit, or dose adjustment is supplied here. Lee has received the sheet but is unclear which number is for routine questions and which route is for urgent concerns. The clinician must clarify the distinction and use teach-back without blame.

PEOPLE / Dr Evans: Cardiologist | Lee: Patient

06

ASK THE PATIENT TO EXPLAIN THE PLAN, NOT PERFORM OBEDIENCE

Teach-back evaluates the clarity of the explanation. A misunderstanding is an opportunity to explain differently, not evidence of poor intelligence or unwillingness.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What has Lee received?

- A A supplied dose-adjustment rule
- B A universal fluid limit
- C A completed follow-up visit
- D A written individualized plan

2. What is unclear?

- A Whether Dr Evans is reviewing it
- B Whether follow-up exists
- C Routine versus urgent contact routes
- D Whether Lee has a plan

3. What does teach-back check?

- A A diagnosis without assessment
- B The clarity of the explanation received
- C The patient's moral character
- D Whether questions should be prohibited

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

heart failure

Clinical syndrome in which cardiac dysfunction causes symptoms and signs in context.

IN USE explain heart failure

congestion

Fluid accumulation associated with relevant circulatory dysfunction.

IN USE assess congestion

edema

Swelling caused by fluid in tissues.

IN USE describe peripheral edema

orthopnea

Breathlessness occurring when lying flat.

IN USE ask about orthopnea

paroxysmal nocturnal dyspnea

Episodes of breathlessness waking a person from sleep.

IN USE clarify nocturnal dyspnea

exercise tolerance

Ability to perform physical activity without limiting symptoms.

IN USE describe exercise tolerance

daily weight record

Log of body-weight measurements under an agreed monitoring plan.

IN USE review a daily weight record

fluid balance

Relationship between fluid intake, output, and retention.

IN USE assess fluid balance

fluid restriction

Individual instruction limiting fluid intake when clinically indicated.

IN USE clarify a prescribed fluid restriction

sodium intake

Amount of sodium consumed through food and drink.

IN USE discuss sodium intake

diuretic

Medicine increasing urine production.

IN USE explain a prescribed diuretic

guideline-directed therapy

Treatment consistent with applicable evidence-based recommendations and individual circumstances.

IN USE review guideline-directed therapy

Precision check

A written action plan does not prove understanding. Clarify the actual instructions; the lesson must not become a universal weight, fluid, or medicine-adjustment protocol.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

decompensation

Worsening of a condition beyond its compensated state.

IN USE assess suspected decompensation

action plan

Individualized instructions about monitoring and responding to changes.

IN USE review the action plan

contact pathway

Defined route for reaching the appropriate care service.

IN USE distinguish contact pathways

self-monitoring

Person's observation or recording of agreed health information.

IN USE explain self-monitoring

follow-up interval

Planned time between reviews in the individual's care.

IN USE confirm the follow-up interval

care-plan discrepancy

Difference between versions or understandings of the intended care plan.

IN USE resolve a care-plan discrepancy

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Routine messaging and urgent care are different routes. Their actual use must be explained for the patient's setting rather than inferred from a worksheet.

Phrases you can actually use

C Notice the language / 1 of 2

CHECK THE EXPLANATION

"I want to check that I explained the two routes clearly."

REMOVE BLAME

"That tells me I need to explain the distinction differently."

SEPARATE ROUTES

"A routine message is not the same as urgent clinical contact."

RESOLVE A MISMATCH

"These two versions need checking before you rely on them."

INVITE TEACH-BACK

"Using your plan, tell me which route you would use for each listed situation."

KEEP IT INDIVIDUAL

"The thresholds and instructions must be the ones agreed for you."

VERIFY THE LIST

"Let us confirm which medicine list is current."

CLOSE WITH CLARITY

"Please show me where the next review and contact details are recorded."

06

I explained

Places responsibility for clear communication on the explanation rather than blaming the listener.

Using your plan

Supports teach-back with the actual written information rather than testing unsupported memory.

The ones agreed for you

Prevents individualized thresholds from being treated as universal instructions.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

06

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which teach-back request is best?

- A** Stop asking because it is printed.
- B** Using your plan, explain which contact route applies to each listed situation.
- C** You understand everything, yes?
- D** Prove you memorized the whole sheet.

2. Which response avoids blame?

- A** The sheet cannot be unclear.
- B** Your question proves you do not care.
- C** I need to explain that distinction differently.
- D** You are bad at following instructions.

3. What must not be invented?

- A** A universal dose-adjustment threshold
- B** A request to verify the plan
- C** A distinction between contact routes
- D** A next review question

4. What does receipt of the sheet establish?

- A** Completed follow-up
- B** An unchanged condition forever
- C** That Lee has the sheet, not that every instruction is understood
- D** Complete comprehension

Answers and explanations: [page 95](#).

Teach-back checks the explanation

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

self-monitoring / action plan / diuretic / contact pathway / fluid restriction / daily weight record / follow-up interval / teach-back / Orthopnea / care-plan discrepancy

06

- 01 **Dr Evans:** You have the written plan. Before you leave, let us check how clearly I explained its different parts.
- 02 **Lee:** I have read the ¹ _____, but the contact numbers still confuse me.
- 03 **Dr Evans:** Thank you for saying so. Tell me which part is unclear, and we can work through that distinction.
- 04 **Lee:** I cannot tell which ² _____ is for ordinary questions and which is for something requiring urgent help.
- 05 **Dr Evans:** We need to make that explicit using your actual instructions. A routine message is not the urgent-care route.
- 06 **Lee:** I also want to understand what I am observing, not just copy numbers into a ³ _____.
- 07 **Dr Evans:** We will explain the monitoring purpose and the instructions agreed for you. This discussion does not supply a universal threshold.
- 08 **Lee:** Good. I have seen different advice about a ⁴ _____, and I did not know whether it applied to everyone.
- 09 **Dr Evans:** It does not automatically apply in the same way to everyone. Use the individualized instructions from your treating team.
- 10 **Lee:** The medicine sheet also needs checking. I do not want to assume an old ⁵ _____ instruction remains current.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

06

- 11 **Dr Evans:** Exactly. We will verify the current list rather than let two different versions sit beside each other unexplained.
- 12 **Lee:** That sounds like a ⁶ _____ we should resolve before I use the sheet at home.
- 13 **Dr Evans:** Yes. Now, using the corrected plan, explain how you would choose the contact route for each situation it lists.
- 14 **Lee:** I can use the written information for that ⁷ _____ plan rather than guessing which changes matter.
- 15 **Dr Evans:** Correct. This ⁸ _____ checks my explanation. A misunderstanding tells me to explain differently, not to blame you.
- 16 **Lee:** I appreciate that. Some symptoms, such as breathlessness when lying flat, are hard to describe with the usual shorthand.
- 17 **Dr Evans:** We can use plain words. ⁹ _____ is the clinical term for that symptom, but your own description remains important.
- 18 **Lee:** I also need to know when the next review is and who answers questions before it.
- 19 **Dr Evans:** We will confirm the ¹⁰ _____ and the relevant contact details in the actual plan.
- 20 **Lee:** Then I can describe the monitoring, medicines, and contact routes separately instead of treating the page as one long instruction.

ANSWERS / EXPLANATIONS: PAGE 96

1. action plan | 2. contact pathway | 3. daily weight record | 4. fluid restriction | 5. diuretic | 6. care-plan discrepancy | 7. self-monitoring | 8. teach-back | 9. Orthopnea | 10. follow-up interval

Two versions of a plan

F Say it

Kai has an old medicine list and a newly printed plan. Their instructions differ. Nurse Jo will obtain clinician verification. No adjustment is authorized by this exercise.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

adjustment / verification / clarified / differ

Kai: "The instructions on the two versions ¹ _____."

Jo: "We need clinician ² _____."

Kai: "This exercise authorizes no medicine ³ _____."

Jo: "The current plan remains to be ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. differ | 2. verification | 3. adjustment | 4. clarified

Rehearse with a fixed script

1. Read the corrected plan review. Give a clear pause between routine questions and urgent concerns.
2. Swap roles. Repeat the teach-back request and the non-blaming clarification without adding clinical thresholds.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Twenty-five percent of what?

A Read the situation

Translate a relative change into comparable absolute frequencies without turning group data into a personal guarantee.

For arithmetic practice only, a fictional comparison describes 40 events among 1,000 people without an intervention and 30 among 1,000 with it over the same five-year period. These are not real treatment estimates or a recommendation. Dr Ali explains the ten-event difference, the one-percentage-point absolute reduction, and the 25 percent relative reduction to Robin. No individual risk estimate is supplied.

07

PEOPLE / Robin: Patient in a fictional numerical exercise | Dr Ali: Cardiologist

KEEP THE DENOMINATOR VISIBLE

A large relative percentage can be heard as a large personal probability change. Use the same group size and time period when comparing figures, then explain the limits of individual prediction.

Briefing checks

Circle one answer in each item. Use the briefing.

1. How many events occur in the fictional comparison group?

- | | |
|---------------------|-----------------------|
| A 10 per 10 | B 40 per 1,000 |
| C 30 per 100 | D 25 per 100 |

2. How many events occur in the intervention group?

- | | |
|-----------------------|-----------------------|
| A 40 per 100 | B 1 per 1,000 |
| C 25 per 1,000 | D 30 per 1,000 |

3. What is the common time period?

- | | |
|----------------------------------|---------------------|
| A An unspecified lifetime | B Five years |
| C One day | D Ten years |

Answers and explanations: [page 97](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

absolute risk

Probability of a defined outcome over a stated time in a specified group.

IN USE communicate absolute risk

relative risk

Ratio of risk in one group to risk in a comparison group.

IN USE calculate relative risk

absolute risk reduction

Difference between comparison and intervention risks.

IN USE explain absolute risk reduction

relative risk reduction

Absolute reduction divided by the comparison risk.

IN USE explain relative risk reduction

percentage point

Unit for the arithmetic difference between two percentages.

IN USE distinguish percentage points

denominator

Total number underlying a proportion or rate.

IN USE state the denominator

natural frequency

Count expressed out of a stated group size.

IN USE use natural frequencies

time horizon

Period over which an outcome probability is described.

IN USE specify the time horizon

comparison group

Group providing a reference for evaluating another group.

IN USE identify the comparison group

intervention group

Group receiving the approach being evaluated.

IN USE describe the intervention group

event rate

Frequency of specified events in a defined group or observation period.

IN USE compare event rates

baseline risk

Risk before or without the evaluated change in the relevant comparison.

IN USE explain baseline risk

Precision check

Forty per 1,000 is 4 percent; thirty per 1,000 is 3 percent. The difference is one percentage point, equivalent to a 25 percent relative reduction from 4 percent.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

confidence interval

Range expressing statistical uncertainty around an estimate under its assumptions.

IN USE interpret a confidence interval

number needed to treat

Reciprocal of absolute risk reduction for a defined outcome and period.

IN USE qualify number needed to treat

population estimate

Estimate describing a defined group rather than a guaranteed individual outcome.

IN USE interpret a population estimate

risk communication

Explanation of probabilities, uncertainty, and possible consequences.

IN USE improve risk communication

benefit framing

Way of presenting a possible favorable outcome or reduction in harm.

IN USE balance benefit framing

uncertainty range

Range communicating limits of an estimate's precision or applicability.

IN USE explain the uncertainty range

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

The figures are invented for language and arithmetic practice. They are not a treatment effect, a personal risk prediction, or evidence for choosing an intervention. Do not invent a confidence interval.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE DENOMINATOR

"Both figures are out of one thousand people."

USE NATURAL FREQUENCIES

"The comparison is forty events versus thirty."

NAME THE ABSOLUTE CHANGE

"The absolute reduction is one percentage point."

LIMIT THE MEANING

"These fictional group figures do not predict your individual outcome."

KEEP TIME CONSISTENT

"Both cover the same five-year period."

EXPLAIN THE DIFFERENCE

"That is ten fewer events per thousand."

NAME THE RELATIVE CHANGE

"Ten is one quarter of forty, so the relative reduction is twenty-five percent."

KEEP UNCERTAINTY HONEST

"No confidence interval has been supplied."

07

Percent versus percentage point

A change from four percent to three percent is one percentage point, not twenty-five percentage points.

Out of

Introduces the denominator in an accessible natural-frequency comparison.

Over five years

Keeps the same observation period attached to both figures.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. What is the absolute difference?

- A 25 percentage points
- B 40 fewer events per 100
- C 10 fewer events per 1,000
- D 250 fewer events per 1,000

2. What is the absolute percentage-point reduction?

- A Seventy-five percentage points
- B One percentage point
- C Twenty-five percentage points
- D Ten percentage points

3. What is the relative reduction?

- A 25 percent
- B 1 percent
- C 75 percent
- D 10 percent

4. Which conclusion must be avoided?

- A These figures guarantee Robin's personal outcome.
- B The denominators match.
- C The time periods match.
- D The figures are fictional.

[Answers and explanations: page 97.](#)

Twenty-five percent of what?

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

denominator / population estimate / percentage point / time horizon / natural frequencies / absolute risk reduction / benefit framing / Relative risk / relative risk reduction / confidence interval

07

- 01 **Robin:** A twenty-five percent reduction sounds large. Does it mean twenty-five fewer people out of every hundred?
- 02 **Dr Ali:** Not in this example. We need the ¹ _____ and the original risk before interpreting a relative percentage.
- 03 **Robin:** Please use the counts first. I follow those more easily than a headline percentage on its own.
- 04 **Dr Ali:** The ² _____ are forty events per thousand without the intervention and thirty per thousand with it.
- 05 **Robin:** Are those counted over the same period, or is one an annual number and the other a lifetime number?
- 06 **Dr Ali:** The ³ _____ is five years for both. We must keep that period attached to the comparison.
- 07 **Robin:** Then the difference is ten events per thousand, rather than two hundred and fifty per thousand.
- 08 **Dr Ali:** Correct. The ⁴ _____ is ten per thousand, which is one percentage point.
- 09 **Robin:** I want to get the wording right: four percent compared with three percent is one percentage point apart.
- 10 **Dr Ali:** Exactly. A ⁵ _____ is the unit for subtracting percentages, not the same as a relative percent change.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Robin:** Where does twenty-five percent come from if the absolute difference is only one percentage point?
- 12 **Dr Ali:** The ⁶ _____ is ten divided by forty. The reduction is one quarter of the comparison count.
- 13 **Robin:** So the relative risk itself is thirty divided by forty, or three quarters of the comparison risk?
- 14 **Dr Ali:** Yes. ⁷ _____ is 0.75 in this fictional example; the relative reduction is 0.25, or twenty-five percent.
- 15 **Robin:** Could someone use only the larger-sounding relative reduction to make the option appear more impressive?
- 16 **Dr Ali:** That is why ⁸ _____ matters. Presenting the absolute figures too helps keep the size of the comparison clear.
- 17 **Robin:** I also understand that these made-up numbers cannot tell me what will happen to me personally.
- 18 **Dr Ali:** Correct. A ⁹ _____ would still need careful interpretation. These made-up figures only illustrate the arithmetic; they are not a prediction for you.
- 19 **Robin:** And we should not add a precision range unless the source actually supplies one and explains it.
- 20 **Dr Ali:** Exactly. No ¹⁰ _____ is supplied here. We should keep that limitation explicit rather than manufacture certainty.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. denominator | 2. natural frequencies | 3. time horizon | 4. absolute risk reduction | 5. percentage point | 6. relative risk reduction | 7. Relative risk | 8. benefit framing | 9. population estimate | 10. confidence interval

A second fictional comparison

F Say it

For arithmetic practice only, fictional five-year event counts are 20 per 1,000 in one group and 15 per 1,000 in another. No treatment recommendation is implied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

0.5 / fictional / twenty-five / five

Clinician: "The absolute difference is ¹ _____ per thousand."

Patient: "That is ² _____ percentage points."

Clinician: "The relative reduction is ³ _____ percent."

Patient: "These numbers remain ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. five | 2. 0.5 | 3. twenty-five | 4. fictional

Rehearse with a fixed script

1. Read the corrected numerical explanation. Stress per thousand, over five years, and percentage point.
2. Swap roles. Say the absolute and relative differences using only the fictional figures supplied.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Referred is not yet enrolled

A Read the situation

Explain a rehabilitation referral and negotiate practical follow-up without giving an unsupplied exercise prescription.

Dr Reed has referred Sam to cardiac rehabilitation after an assessed cardiac event. The rehabilitation team has received the referral, but no intake appointment is confirmed. Sam wants to return to a physically demanding job and has a transport barrier. Exercise clearance, work restrictions, and an individual program require the actual clinical assessment; none is supplied in this exercise. Coordinator Inez will help clarify access arrangements.

08

PEOPLE / Sam: Patient | Dr Reed: Cardiologist

PRACTICAL BARRIERS ARE PART OF THE CLINICAL CONVERSATION

Attendance can be difficult even when the patient wants support. Ask about transport and work demands before labeling missed access as a lack of motivation.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is confirmed?

- | | |
|---|--|
| A Receipt of the rehabilitation referral | B A completed intake |
| C Full work clearance | D A prescribed exercise intensity |

2. What barrier does Sam report?

- | | |
|------------------------------------|--|
| A Transport | B A refusal of all rehabilitation |
| C A completed exercise test | D A supplied work restriction |

3. What requires individual assessment?

- | | |
|--------------------------------------|-------------------------------------|
| A Sam's expressed wish | B Exercise and work guidance |
| C Whether a referral was sent | D Inez's name |

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

cardiac rehabilitation

Supervised program supporting recovery, risk reduction, and functioning after eligible cardiac conditions.

IN USE explain cardiac rehabilitation

rehabilitation intake

Initial assessment and planning encounter for a rehabilitation program.

IN USE arrange rehabilitation intake

exercise prescription

Individualized specification of appropriate exercise under clinical assessment.

IN USE review an exercise prescription

functional capacity

Ability to perform physical tasks under relevant conditions.

IN USE assess functional capacity

graded activity

Activity progressed in steps under an appropriate plan.

IN USE discuss graded activity

supervised exercise

Exercise performed with the relevant professional oversight.

IN USE arrange supervised exercise

return-to-work assessment

Evaluation of readiness and requirements for resuming work.

IN USE request a return-to-work assessment

work restriction

Specified limitation on work activity under an appropriate assessment.

IN USE clarify work restrictions

occupational demand

Physical or other requirement of a person's job.

IN USE describe occupational demands

secondary prevention

Measures reducing recurrence or complications after established disease.

IN USE discuss secondary prevention

risk-factor modification

Changes directed at relevant disease risk factors.

IN USE support risk-factor modification

rehabilitation goal

Individual objective for recovery or functioning.

IN USE set a rehabilitation goal

Precision check

A rehabilitation referral is not an exercise prescription or work-clearance certificate. Keep acceptance, intake, assessment, and participation distinct.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

attendance barrier

Obstacle to taking part in scheduled care.

IN USE identify an attendance barrier

transport arrangement

Plan for travel to and from a service.

IN USE confirm transport arrangements

program eligibility

Whether a person meets requirements for a particular service.

IN USE verify program eligibility

coverage verification

Checking the applicable financial or insurance arrangements.

IN USE obtain coverage verification

intake confirmation

Verification that an initial program appointment is booked.

IN USE obtain intake confirmation

care coordinator

Professional organizing connected services and practical arrangements.

IN USE contact the care coordinator

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

08

Precision check

Do not promise coverage, transport, or a place in a program before verification. Help identify the responsible person and the unresolved practical questions.

Phrases you can actually use

C Notice the language / 1 of 2

EXPLAIN THE SERVICE

"Rehabilitation combines assessed activity, education, and support."

ASK ABOUT ACCESS

"What would make getting to the sessions difficult?"

AVOID CLEARANCE BY IMPLICATION

"This referral is not a statement that every work task is safe for you."

SEPARATE COVERAGE

"We need to verify the applicable coverage rather than promise it."

STATE THE STAGE

"The team has received the referral; intake is not yet confirmed."

DESCRIBE THE JOB

"Tell us what the work requires physically."

NAME THE COORDINATOR

"Inez can help clarify the access arrangements."

CONFIRM THE NEXT STEP

"Let us identify who will confirm the intake and contact you."

08

Received versus confirmed

Receipt concerns the referral; confirmed intake concerns a booked appointment.

Requires physically

Elicits actual job demands instead of relying on a job title alone.

Can help clarify

Offers coordination without guaranteeing a service or financial outcome.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement preserves the referral stage?

- A** Received, but intake not yet confirmed.
- B** Sam is already enrolled.
- C** Full work clearance has been issued.
- D** The exercise program is finalized.

2. Which question addresses access respectfully?

- A** Why do you refuse all care?
- B** What makes travel to the sessions difficult?
- C** Why are you unmotivated?
- D** Can we ignore transport?

3. What must not be inferred from the referral?

- A** A safe exercise intensity or unrestricted work clearance
- B** A need to clarify intake
- C** A transport question
- D** A coordinator's role

4. Which promise is unsupported?

- A** Coverage needs verification.
- B** Every cost and journey will definitely be covered.
- C** Inez can help clarify arrangements.
- D** Intake is not confirmed.

Answers and explanations: page 99.

Referred is not yet enrolled

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

graded activity / occupational demands / return-to-work assessment / rehabilitation goal / coverage verification / exercise prescription / Cardiac rehabilitation / attendance barrier / intake confirmation / care coordinator

08

- 01 **Sam:** I want to return to work, but the job involves lifting and long periods on my feet.
- 02 **Dr Reed:** Those ¹ _____ need to be described for the assessment rather than inferred from your job title.
- 03 **Sam:** Does the rehabilitation referral mean I have already been cleared to do every part of the job?
- 04 **Dr Reed:** No. A ² _____ is a separate clinical question. The referral is not unrestricted clearance.
- 05 **Sam:** Then what is rehabilitation intended to add beyond another appointment about the event itself?
- 06 **Dr Reed:** ³ _____ can combine assessed activity, education, and support, with a program suited to the relevant clinical circumstances.
- 07 **Sam:** I received a letter saying the referral arrived. I assumed that meant I had a first session booked.
- 08 **Dr Reed:** The team has received it, but ⁴ _____ has not yet been provided. We need to make that next step clear.
- 09 **Sam:** Getting there may be difficult. I do not have a reliable lift during the hours the service usually operates.
- 10 **Dr Reed:** That ⁵ _____ matters. Inez can help clarify transport and access arrangements rather than assume you do not want to attend.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Sam:** I also need to know whether the sessions are covered before I commit to costs I cannot manage.
- 12 **Dr Reed:** We need ⁶ _____. I cannot promise a financial outcome without checking the applicable arrangements.
- 13 **Sam:** A friend suggested copying their exercise schedule while I wait. Is that the same as the program I will receive?
- 14 **Dr Reed:** No. An ⁷ _____ needs your own assessment. Ask the rehabilitation team what is appropriate for you rather than copying your friend's schedule.
- 15 **Sam:** I would like the program to help me with the tasks that actually matter at home and work.
- 16 **Dr Reed:** That is a useful ⁸ _____ to discuss with the team as they assess your circumstances.
- 17 **Sam:** I understand that progress may involve different steps rather than an immediate return to everything.
- 18 **Dr Reed:** Yes. Any ⁹ _____ must follow the agreed clinical plan, with the relevant review and support.
- 19 **Sam:** For now I need the intake date, transport information, and a clear route for my work questions.
- 20 **Dr Reed:** We will identify the ¹⁰ _____ and responsible clinicians for those questions, without treating the referral as a completed assessment.

08

ANSWERS / EXPLANATIONS: PAGE 100

1. occupational demands | 2. return-to-work assessment | 3. Cardiac rehabilitation | 4. intake confirmation | 5. attendance barrier | 6. coverage verification | 7. exercise prescription | 8. rehabilitation goal | 9. graded activity | 10. care coordinator

A job title hides the task

F Say it

Rory works in a warehouse and asks about returning to lifting. The clinical team requests the actual task demands before assessing restrictions. No lifting limit is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

lifting / assessment / supplied / demands

08

Rory: "My job includes ¹ _____."

Clinician: "We need the task ² _____ before assessing restrictions."

Rory: "No lifting limit has been ³ _____."

Clinician: "The work guidance needs individual ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. lifting | 2. demands | 3. supplied | 4. assessment

Rehearse with a fixed script

1. Read the corrected rehabilitation discussion. Contrast referral receipt, intake, and work assessment.
2. Swap roles. Repeat the transport-barrier question without implying poor motivation or promising coverage.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

An alert from a cardiac device

E More workplace conversations / 1-10

Distinguish a device transmission, an alert, and a reviewed clinical finding.

Patient Ellis sees a message that a device transmission was received and asks whether it proves a new problem. Dr Kim explains the actual review process. No alert type, device setting, symptom assessment, or diagnosis is supplied. Device messaging is not a substitute for the actual urgent-care route if concerning symptoms occur.

PEOPLE / Ellis: Patient | Dr Kim: Cardiologist

Use each bank entry once, without changing its form. Answers follow turn 20.

09

WORD + PHRASE BANK

Data availability / review workflow / urgent-care instructions / alert / device transmission / review status

- 01 **Ellis:** My device account says a transmission was received. Does that mean the team has found a new heart problem?
- 02 **Dr Kim:** Receipt confirms that information arrived. It does not, by itself, tell us the clinical interpretation.
- 03 **Ellis:** I thought every notification meant a clinician had already reviewed the recording and reached a conclusion.
- 04 **Dr Kim:** A ¹ _____ and a reviewed finding are separate stages.
- 05 **Ellis:** What about the word alert? Is that always a diagnosis rather than a signal for someone to check?
- 06 **Dr Kim:** An ² _____ needs the actual system context and review; the word alone does not establish a diagnosis.
- 07 **Ellis:** I do not want to ignore something important, but I also do not want to invent a conclusion.
- 08 **Dr Kim:** We should clarify the real ³ _____ and who communicates the outcome, without promising a finding before review.
- 09 **Ellis:** Does this kind of system always mean someone is watching every heartbeat in real time?
- 10 **Dr Kim:** Do not assume that. The service must explain the actual monitoring arrangements and their limitations for your device.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Ellis:** If I have a concerning new symptom, should I just send data and wait for the next message?
- 12 **Dr Kim:** Follow the actual ⁴ _____ rather than treating a routine transmission as a substitute for assessment.
- 13 **Ellis:** I also want to know how to report a problem with the connection itself.
- 14 **Dr Kim:** The team can explain the appropriate technical contact and distinguish a connection problem from a clinical finding.
- 15 **Ellis:** So a missing signal would not automatically prove that my heart rhythm is normal or abnormal?
- 16 **Dr Kim:** Correct. ⁵ _____ is a separate question from what the information means clinically.
- 17 **Ellis:** Could we confirm who explains the reviewed result and how I can check the status?
- 18 **Dr Kim:** Yes. We will identify the actual contact route and distinguish a pending review from a completed discussion.
- 19 **Ellis:** That helps me understand the message without mistaking an administrative confirmation for a diagnosis.
- 20 **Dr Kim:** A clear ⁶ _____ prevents exactly that confusion while keeping the appropriate clinical contact route visible.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. device transmission | 2. alert | 3. review workflow | 4. urgent-care instructions | 5. Data availability | 6. review status

The data arrived but review is pending

F Say it in a new situation

The service confirms receipt of a transmission. Clinical review is pending. The patient has not reported a symptom in this update.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

status / pending / received / diagnosis

09

Patient: "The transmission was ¹ _____."

Clinician: "Clinical review remains ² _____."

Patient: "That is not a confirmed ³ _____."

Clinician: "We should clarify the review ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 101

1. received | 2. pending | 3. diagnosis | 4. status

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Reconciling a cross-specialty medicine plan

E More workplace conversations / 1-10

Resolve conflicting instructions through the responsible clinicians without independently changing treatment.

Dr Reed, a cardiologist, and Dr Hall, a procedural clinician, find conflicting medicine instructions in two appointment documents. The actual medicines, procedure, and doses are not supplied. They agree to verify the current plan, responsible prescriber, and patient communication before presenting an instruction as settled.

PEOPLE / Dr Reed: Cardiologist | Dr Hall: Procedural clinician

Use each bank entry once, without changing its form. Answers follow turn 20.

10

WORD + PHRASE BANK

responsible prescriber / closed-loop reconciliation / communication ownership / reconciled instruction / procedural context / document version

- 01 **Dr Hall:** Our appointment document contains a medicine instruction that conflicts with the cardiology letter. The patient has noticed the difference.
- 02 **Dr Reed:** We need to resolve that through the responsible clinicians, not ask the patient to guess which document to follow.
- 03 **Dr Hall:** I will identify the exact instruction and date rather than summarize it as a general disagreement.
- 04 **Dr Reed:** The ¹ _____ matters. We should establish which copies are current and what authority each instruction has.
- 05 **Dr Hall:** The procedure team needs the actual clinical context, not an assumption that all procedures use identical instructions.
- 06 **Dr Reed:** Agreed. The ² _____ must be verified before a medicine plan is treated as appropriate.
- 07 **Dr Hall:** We should also identify the clinician responsible for the prescription rather than assume the appointment office owns the decision.
- 08 **Dr Reed:** Let us confirm the ³ _____ and the necessary cross-specialty discussion.
- 09 **Dr Hall:** Until then, I will not describe a stop or restart instruction as agreed merely because it appears in one letter.
- 10 **Dr Reed:** Correct. We are reconciling conflicting information, not issuing a new medicine instruction in this conversation.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 Dr Hall:** The patient needs a single clear explanation once the actual decision has been verified.
- 12 Dr Reed:** A ⁴ _____ should identify the relevant medicine details, timing, and contact route in the real clinical plan.
- 13 Dr Hall:** If one document is superseded, should we retain the correction history rather than silently erase the old wording?
- 14 Dr Reed:** Yes. Follow the actual record-correction process so the change and its authority remain traceable.
- 15 Dr Hall:** I will confirm who contacts the patient and how we verify that the conflicting message has been corrected.
- 16 Dr Reed:** That is ⁵ _____, which must not disappear after the clinicians agree among themselves.
- 17 Dr Hall:** We should also update the relevant teams so the old instruction is not repeated at the next appointment.
- 18 Dr Reed:** Exactly. A resolved discussion is incomplete if the outdated message remains the one everyone keeps using.
- 19 Dr Hall:** I will record the verified plan, the responsible clinicians, and the communication outcome under our actual process.
- 20 Dr Reed:** Then ⁶ _____ includes both a clinical decision and accurate communication to the people who need it.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. document version | 2. procedural context | 3. responsible prescriber | 4. reconciled instruction | 5. communication ownership | 6. closed-loop reconciliation

The old letter is still being used

F Say it in a new situation

The clinicians have verified the current plan, but an appointment office is still sending the older letter. The team requests an authorized correction and patient communication.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

communication / correction / older / verified

Clinician: "The current plan is ¹ _____."

Office: "We are still sending the ² _____ letter."

Clinician: "We need an authorized ³ _____."

Office: "The patient also needs clear ⁴ _____."

10

ANSWERS / EXPLANATIONS: PAGE 102

1. verified | 2. older | 3. correction | 4. communication

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Discussing priorities in advanced heart disease

E More workplace conversations / 1-10

Elicit care goals without equating supportive care with abandonment or a predetermined resuscitation decision.

Patient Avery, living with advanced heart disease, wants less breathlessness and more time at home. Dr Shah explores those priorities and explains that supportive or palliative care can be discussed alongside disease-directed care. No prognosis estimate, treatment withdrawal, or resuscitation decision is supplied.

PEOPLE / Avery: Patient | Dr Shah: Cardiologist

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

symptom burden / care goals / advance care discussion / care coordinator / Palliative care / treatment burden

11

- 01 **Avery:** I want less breathlessness and more time at home. I am worried that saying that sounds like giving up.
- 02 **Dr Shah:** Those priorities deserve a direct discussion. Telling us what matters does not automatically mean stopping disease-directed care.
- 03 **Avery:** I would like the team to hear the practical things I am finding hardest, not only the test results.
- 04 **Dr Shah:** We can discuss your ¹ _____ and how it affects the activities and time that matter to you.
- 05 **Avery:** Someone mentioned palliative care, and I thought that meant all other treatment would end immediately.
- 06 **Dr Shah:** ² _____ can be discussed alongside other treatment. It does not automatically mean that every treatment stops.
- 07 **Avery:** I also worry that a conversation about priorities will be recorded as a decision about resuscitation.
- 08 **Dr Shah:** A discussion of ³ _____ is not itself a resuscitation decision. Any specific decision needs its own appropriate discussion.
- 09 **Avery:** I would like my daughter involved, but I still want to speak for myself while I can.
- 10 **Dr Shah:** We can include her with your permission while keeping you central to the conversation and clarifying the relevant roles.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Avery:** If there are options with different burdens, I want those explained in terms I can compare.
- 12 **Dr Shah:** We should discuss ⁴ _____ alongside expected benefits, using the actual clinical options rather than an invented choice.
- 13 **Avery:** Can you guarantee that choosing a particular plan will keep me out of hospital?
- 14 **Dr Shah:** No. We can explain the aims and uncertainties, but we should not promise an outcome that cannot be guaranteed.
- 15 **Avery:** Then I would like a plan that records my priorities without pretending the future is settled.
- 16 **Dr Shah:** An ⁵ _____ can help, with the actual documentation and legal arrangements explained where relevant.
- 17 **Avery:** What happens if my priorities change as my symptoms or circumstances change?
- 18 **Dr Shah:** We should revisit them. A record should support your care, not freeze a preference that no longer represents you.
- 19 **Avery:** I would appreciate a clear contact who can help coordinate the clinical and practical support.
- 20 **Dr Shah:** We can identify the appropriate ⁶ _____ and confirm the next discussion without converting today's priorities into an unmade treatment decision.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. symptom burden | 2. Palliative care | 3. care goals | 4. treatment burden | 5. advance care discussion | 6. care coordinator

A preference has changed

F Say it in a new situation

A patient previously prioritized clinic-based treatment but now asks to discuss more care at home. No treatment change has yet been agreed.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

options / heard / agreed / priorities

Patient: "I want to revisit my ¹ _____."

Clinician: "A treatment change is not yet ² _____."

Patient: "My current wishes should be ³ _____."

Clinician: "We will explain the actual ⁴ _____."

11

ANSWERS / EXPLANATIONS: PAGE 103

1. priorities | 2. agreed | 3. heard | 4. options

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Giving an urgent cardiovascular handoff

A Briefing checks / page 4

1. What symptoms are reported?

C. Chest pressure with breathlessness

The case supplies the symptom report without establishing its cause.

2. What is already active?

D. The emergency response

The emergency response begins before the communication exercise.

3. Who accepts the handoff?

C. Dr Chen

Dr Chen is the receiving cardiologist explicitly accepting the report.

D Language checks / page 9

1. Which opening is strongest?

A. Chest pressure with breathlessness; the emergency response is active.

Naming the symptoms and active response communicates the actual concern and action without adding unsupported findings.

2. Which result must not be invented?

B. A normal troponin

No troponin result is supplied, so normal cannot be inserted as reassurance.

3. What completes the communication loop?

D. Confirmation of the receiving team and responsibility

Closed-loop communication requires acknowledgment rather than merely attempted contact.

4. What should not delay emergency care?

C. Completing every detail of this language exercise

The exercise is for language practice and must never replace or delay the real response.

01 Conversation answers

Giving an urgent cardiovascular handoff

E Dialogue gaps

Pages 10-11

- 1. time-critical concern.** Turn 2. A time-critical concern requires the active clinical response while the handoff conveys known facts and unresolved information.
- 2. dyspnea.** Turn 4. Dyspnea is the reported breathing difficulty; the term does not by itself establish a cause or measurement.
- 3. hemodynamic status.** Turn 6. Hemodynamic status requires relevant circulatory assessment and cannot be called normal from missing measurements.
- 4. prehospital report.** Turn 8. The prehospital report contains information collected before hospital arrival and needs accurate source and timing attribution.
- 5. Myocardial infarction.** Turn 10. Myocardial infarction is a diagnosis requiring appropriate evidence and is not established by the supplied symptom report alone.
- 6. troponin.** Turn 12. Troponin is interpreted as a cardiac-injury biomarker with timing and context; no value is supplied in this case.
- 7. receiving team.** Turn 14. The receiving team accepts the next stage of care and needs updates through the active clinical channel.
- 8. Emergency activation.** Turn 16. Emergency activation initiates the relevant urgent response and must not be delayed while a language report is perfected.
- 9. closed-loop escalation.** Turn 18. Closed-loop escalation includes acknowledgment and accepted responsibility, not simply sending information without confirmation.
- 10. symptom trajectory.** Turn 20. Symptom trajectory describes how symptoms change over time and should be reported accurately as new information becomes available.

F Transfer conversation

Page 12

1. 09:10

Referrer: "The corrected reported onset is 09:10."

The patient corrects the reported onset to 09:10 in the supplied facts.

2. 09:40

Receiver: "The earlier time of 09:40 is superseded."

The earlier report is corrected rather than retained as the current onset.

3. active

Referrer: "The emergency response remains active."

Correcting the history does not interrupt the active emergency response.

4. unestablished

Receiver: "The cause remains unestablished."

The timing correction does not itself establish a diagnosis.

02 Briefing and language checks

Explaining ambulatory rhythm monitoring

A Briefing checks / page 13

1. What is the proposed purpose?

C. Compare symptoms with recorded rhythm

The proposed recording explores symptom-rhythm relationships without a guaranteed diagnosis.

2. What is not established?

A. An arrhythmia

The case supplies palpitations but no confirmed rhythm diagnosis.

3. Who provides equipment instructions?

A. The service using the actual device

Instructions must match the actual device and service, not assumptions in the exercise.

D Language checks / page 18

1. Which distinction is correct?

A. Palpitations are a symptom; an arrhythmia is a rhythm abnormality.

The symptom and the electrical finding require correlation rather than automatic equivalence.

2. What makes a diary entry useful?

A. Time, sensation, and activity

The relevant details help connect a reported event with the corresponding recorded interval.

3. Which promise is unsupported?

B. Every episode will definitely be captured.

A finite recording may not capture every intermittent symptom or produce a definitive explanation.

4. What must remain separate?

A. Urgent-care instructions and routine monitor review

Routine recording does not replace appropriate urgent assessment of concerning clinical changes.

02 Conversation answers

Explaining ambulatory rhythm monitoring

E Dialogue gaps

Pages 19-20

- 1. palpitations.** Turn 2. Palpitations describe the awareness of an unusual heartbeat and do not by themselves identify a specific rhythm abnormality.
- 2. ambulatory monitor.** Turn 4. An ambulatory monitor records rhythm during everyday activity and may help investigate symptoms that are intermittent.
- 3. arrhythmia.** Turn 6. An arrhythmia is an abnormal heart rhythm, which has not been established by this symptom account.
- 4. symptom diary.** Turn 8. A symptom diary records the sensation, timing, and activity so the reviewing clinician can interpret events in context.
- 5. event marker.** Turn 10. An event marker identifies a time for subsequent review and does not itself diagnose the event.
- 6. symptom-rhythm correlation.** Turn 12. Symptom-rhythm correlation compares the reported experience with the recorded rhythm at the relevant time.
- 7. Signal artifact.** Turn 14. Signal artifact is distortion that may arise during recording and must be distinguished from the underlying cardiac signal.
- 8. signal quality.** Turn 16. Signal quality concerns whether the recording is adequate for interpretation and depends on the actual equipment and circumstances.
- 9. recording period.** Turn 18. The recording period is the data-collection interval and is not a reason to defer necessary urgent clinical assessment.
- 10. monitor return.** Turn 20. Monitor return is the equipment or data-return process needed for review and should not be left as an assumed final step.

F Transfer conversation

Page 21

1. evening

Reviewer: "The entry gives seven but not morning or evening."

The time is incomplete because its morning-or-evening context is missing.

2. clarify

Ellis: "I need to clarify that detail."

Clarification is required before assigning the diary entry to a recording interval.

3. trace

Reviewer: "Then we can match it to the trace."

The purpose is to compare the clarified symptom time with the rhythm recording.

4. supplied

Ellis: "No rhythm conclusion has been supplied."

The scenario provides a data clarification, not a diagnosed rhythm.

03 Briefing and language checks

Explaining an echocardiogram

A Briefing checks / page 22

1. What does Ren misunderstand?

D. The meaning of the ejection-fraction percentage

Ren thinks the percentage measures remaining living heart muscle, which is not what it describes.

2. What is not supplied?

D. A numerical value or heart-failure diagnosis

The exercise provides a terminology discussion without an individual value or diagnosis.

3. What else belongs in interpretation?

C. Symptoms and other cardiac findings

The brief explicitly places the measurement within a broader clinical and imaging assessment.

D Language checks / page 27

1. What does ejection fraction measure?

C. A proportion of ventricular blood expelled with contraction

The measurement concerns pumping from a ventricle, not tissue survival or a global health percentage.

2. Which statement is accurate?

A. Preserved ejection fraction alone does not exclude heart failure.

Heart failure and cardiac function require the full clinical and imaging context.

3. Which distinction is useful?

D. Stroke volume is an amount; ejection fraction is a proportion.

The terms describe related but different ways of expressing ventricular ejection.

4. Which conclusion exceeds this case?

C. Ren has a specific heart-failure category.

The scenario intentionally supplies neither an individual measurement nor a diagnosis.

03 Conversation answers

Explaining an echocardiogram

E Dialogue gaps

Pages 28-29

1. Ejection fraction. Turn 2. Ejection fraction is the proportion of ventricular blood expelled with a contraction, not the proportion of living heart tissue.

2. left ventricle. Turn 4. The left ventricle is the chamber pumping blood into the body's systemic circulation and is the chamber named in this report.

3. ventricular volume. Turn 6. Ventricular volume is the blood in the chamber at a specified phase and provides the relevant basis for the pumping proportion.

4. Stroke volume. Turn 8. Stroke volume is the amount ejected in one beat, which differs from expressing that ejection as a proportion.

5. Systole. Turn 10. Systole includes ventricular contraction and ejection and is distinct from the relaxation and filling phase.

6. Diastolic function. Turn 12. Diastolic function concerns relaxation and filling and can matter even when a pumping proportion appears preserved.

7. Preserved ejection fraction. Turn 14. Preserved ejection fraction describes a measurement category and does not by itself exclude heart failure or other dysfunction.

8. valvular regurgitation. Turn 16. Valvular regurgitation means backward flow through a valve and is not synonymous with an ejection-fraction measurement.

9. image quality. Turn 18. Image quality affects what an examination can establish and relevant limitations should remain visible in the interpretation.

10. comparison study. Turn 20. A comparison study is an earlier examination used to assess change and must be interpreted for comparability and context.

F Transfer conversation

Page 30

1. scan

Patient: "The report refers to an earlier scan."

An earlier examination is mentioned as a potential comparison.

2. comparable

Clinician: "I still need to review whether they are comparable."

Comparability is unresolved and cannot be assumed from the existence of two reports.

3. established

Patient: "A meaningful change is not established."

The case supplies no completed interpretation of a change.

4. context

Clinician: "We will interpret the findings in context."

The relevant clinical and examination context must accompany the comparison.

04 Briefing and language checks

Discussing atrial-fibrillation treatment goals

04

A Briefing checks / page 31

1. What diagnosis is established?

B. Atrial fibrillation

The brief explicitly identifies atrial fibrillation as an established diagnosis.

2. What does Avery ask about?

D. Why stroke prevention matters when symptoms feel calmer

Avery links improved sensations with a question about continuing risk discussion.

3. What is absent from the case?

B. A medicine selection or risk score

No individual score, prescription, dose, or stopping instruction may be invented.

D Language checks / page 36

1. Which distinction is correct?

A. Rhythm or symptom control does not replace the stroke-prevention assessment.

Treatment goals overlap but require their own relevant clinical assessment.

2. Which question includes Avery's concern?

A. What previous bleeding have you experienced, and what worries you?

Asking about previous bleeding and worries elicits relevant history without minimizing the concern.

3. Which instruction cannot be supplied here?

A. Stop anticoagulation because the heartbeat feels calmer.

The case gives no medication-change instruction or evidence for making one.

4. Why clarify blood thinner?

D. The informal phrase can hide different medicine actions.

The phrase is colloquial and does not distinguish anticoagulants from other therapies.

04 Conversation answers

Discussing atrial-fibrillation treatment goals

E Dialogue gaps

Pages 37-38

1. atrial fibrillation. Turn 2. Atrial fibrillation is the established rhythm diagnosis here, and its care includes distinct symptom and stroke-risk questions.

2. stroke risk. Turn 4. Stroke risk requires individual assessment and cannot be assumed absent merely because palpitations feel less noticeable.

3. Rate control. Turn 6. Rate control aims at controlling heart rate and is distinct from a strategy focused on restoring or maintaining rhythm.

4. bleeding risk. Turn 8. Bleeding risk is a relevant part of the individualized benefit-and-harm discussion and should not be dismissed.

5. Anticoagulation. Turn 10. Anticoagulation acts on coagulation pathways and is not simply another name for every medicine informally called a blood thinner.

6. bleeding history. Turn 12. Bleeding history records previous events and their circumstances and is relevant to an individualized clinical review.

7. drug interaction. Turn 14. A drug interaction can change a medicine's effects and belongs in review of the actual medicines and clinical context.

8. individual risk estimate. Turn 16. An individual risk estimate depends on the relevant person, outcome, period, and clinical information, not a generic online chart alone.

9. shared treatment decision. Turn 18. A shared treatment decision combines clinical evidence with the person's informed priorities rather than ignoring either.

10. anticoagulation review. Turn 20. An anticoagulation review reassesses the actual treatment and risks; the language exercise supplies no authorization to change a medicine.

F Transfer conversation

Page 39

1. name

Nico: "I do not recall the product name."

The medicine's identity remains unresolved despite the informal label.

2. purpose

Clinician: "We will verify its name and purpose."

Verification must establish what the actual medicine is intended to do.

3. given

Nico: "No stop instruction has been given."

The case provides no instruction to discontinue the medicine.

4. review

Clinician: "A change needs individual review."

An individual clinical review is required before deciding on a change.

04

05 Briefing and language checks

Discussing coronary angiography consent

A Briefing checks / page 40

1. What is proposed?

B. Diagnostic coronary angiography

The case proposes a diagnostic investigation, not a settled intervention.

2. What does Blair assume?

C. A stent will definitely be inserted

Blair confuses the proposed diagnostic procedure with an automatic treatment decision.

3. What is not supplied?

D. Coronary anatomy or an intervention decision

The clinician cannot invent anatomical findings or a final intervention plan.

D Language checks / page 45

1. Which distinction is essential?

D. Diagnostic angiography does not automatically mean stent insertion.

The proposed diagnostic purpose and any intervention agreement must be distinguished.

2. Which instruction would be inappropriate to invent?

B. A medication-holding schedule

Preparation and medicine instructions must come from the actual clinical team.

3. What does informed agreement require here?

B. A clear discussion of the specified procedure and relevant options

Consent concerns an understood, specified proposal and cannot be replaced by attendance alone.

4. Which statement preserves the facts?

A. No intervention decision is established in this case.

The scenario does not provide findings or a decision authorizing intervention.

05 Conversation answers

Discussing coronary angiography consent

E Dialogue gaps

Pages 46-47

1. coronary angiography. Turn 2. Coronary angiography examines coronary arteries; it does not automatically include stent treatment.

2. diagnostic procedure. Turn 4. A diagnostic procedure obtains information and must be distinguished from a separate treatment action or decision.

3. coronary anatomy. Turn 6. Coronary anatomy describes the arteries; this case supplies no anatomical findings.

4. Percutaneous coronary intervention. Turn 8. Percutaneous coronary intervention is catheter-based treatment, distinct from diagnostic imaging alone.

5. procedure consent. Turn 10. Procedure consent concerns the specified actions agreed after discussion, including any possible intervention in the actual plan.

6. stent. Turn 12. A stent is a device used in some interventions and is not automatically inserted merely because angiography is discussed.

7. vascular access. Turn 14. Vascular access identifies the actual vessel-entry route, not a plan borrowed from another patient.

8. contrast medium. Turn 16. Contrast medium improves imaging visibility and its relevant risks and preparation need individual clinical review.

9. preparation instructions. Turn 18. Preparation instructions must match the real procedure and medicines; the language exercise does not supply a safe substitute.

10. postprocedure observation. Turn 20. Postprocedure observation means planned monitoring; it does not establish a guaranteed discharge time.

F Transfer conversation

Page 48

1. instructions

Morgan: "The two sheets give different instructions."

The problem is conflicting preparation information rather than a completed clinical decision.

2. verify

Clinician: "The procedural team will verify which applies."

Verification by the responsible team is the supplied next step.

3. authorized

Morgan: "No medicine change is authorized here."

The scenario supplies no authorization to alter a medicine.

4. clarified

Clinician: "The correct plan is not yet clarified."

Conflicting documents leave the relevant instruction unresolved until verification.

05

06 Briefing and language checks

Checking a heart-failure home plan

A Briefing checks / page 49

1. What has Lee received?

D. A written individualized plan

The sheet exists, but this exercise supplies no clinical thresholds or dosing rules.

2. What is unclear?

C. Routine versus urgent contact routes

Lee needs the two communication routes distinguished explicitly.

3. What does teach-back check?

B. The clarity of the explanation received

Teach-back reveals how the explanation was understood and supports clarification.

D Language checks / page 54

1. Which teach-back request is best?

B. Using your plan, explain which contact route applies to each listed situation.

Explaining the appropriate contacts reveals understanding of the plan instead of merely obtaining a yes-or-no answer.

2. Which response avoids blame?

C. I need to explain that distinction differently.

Teach-back should prompt clearer explanation rather than criticism of the patient.

3. What must not be invented?

A. A universal dose-adjustment threshold

The exercise supplies no numerical or medicine-change instructions to generalize.

4. What does receipt of the sheet establish?

C. That Lee has the sheet, not that every instruction is understood

Possession of a document is different from understanding or completion of care.

06 Conversation answers

Checking a heart-failure home plan

E Dialogue gaps

Pages 55-56

- 1. action plan.** Turn 2. An action plan provides individualized monitoring and response instructions, but receiving it does not prove complete understanding.
- 2. contact pathway.** Turn 4. A contact pathway identifies the appropriate route to a service; routine questions and urgent concerns use different actual arrangements.
- 3. daily weight record.** Turn 6. A daily weight record is an agreed monitoring log and does not itself supply a universal threshold for action.
- 4. fluid restriction.** Turn 8. A fluid restriction is an individual clinical instruction when indicated, not a rule that can be borrowed from another person's plan.
- 5. diuretic.** Turn 10. A diuretic is a medicine that increases urine production; its actual instructions need verification rather than guesswork.
- 6. care-plan discrepancy.** Turn 12. A care-plan discrepancy is a difference between versions or understandings that requires verification before reliance.
- 7. self-monitoring.** Turn 14. Self-monitoring means observing or recording agreed information and should be linked to the actual individualized response plan.
- 8. teach-back.** Turn 15. Teach-back checks how clearly the explanation was understood and should prompt clarification rather than blame.
- 9. Orthopnea.** Turn 17. Orthopnea is breathlessness when lying flat; explaining the term does not replace assessment of the person's actual symptoms.
- 10. follow-up interval.** Turn 19. The follow-up interval is the individually planned time between reviews and must not be guessed from a generic worksheet.

F Transfer conversation

Page 57

1. differ

Kai: "The instructions on the two versions differ."

The discrepancy concerns conflicting instructions rather than a confirmed current plan.

2. verification

Jo: "We need clinician verification."

The responsible clinical review must establish which instructions apply.

3. adjustment

Kai: "This exercise authorizes no medicine adjustment."

No medicine change may be inferred from the language-practice scenario.

4. clarified

Jo: "The current plan remains to be clarified."

The supplied next step is resolving the discrepancy before relying on a version.

07 Briefing and language checks

Explaining absolute and relative risk

A Briefing checks / page 58

1. How many events occur in the fictional comparison group?

B. 40 per 1,000

The supplied comparison group has forty events among one thousand people over five years.

2. How many events occur in the intervention group?

D. 30 per 1,000

The intervention group has thirty events among the same denominator of one thousand.

3. What is the common time period?

B. Five years

Both fictional figures refer to the same five-year period for a valid comparison.

D Language checks / page 63

1. What is the absolute difference?

C. 10 fewer events per 1,000

Subtracting thirty from forty gives ten fewer events among one thousand people.

2. What is the absolute percentage-point reduction?

B. One percentage point

Four percent minus three percent equals one percentage point.

3. What is the relative reduction?

A. 25 percent

The reduction of ten divided by the comparison count of forty is one quarter, or twenty-five percent.

4. Which conclusion must be avoided?

A. These figures guarantee Robin's personal outcome.

Fictional group data cannot establish a personal risk or guarantee an individual result.

07 Conversation answers

Explaining absolute and relative risk

E Dialogue gaps

Pages 64-65

- 1. denominator.** Turn 2. The denominator is the group size underlying each figure; both fictional groups contain one thousand people.
- 2. natural frequencies.** Turn 4. Natural frequencies express the counts out of a common group size, here forty versus thirty events per thousand.
- 3. time horizon.** Turn 6. The time horizon is the period over which the event figures apply; both use the same five years.
- 4. absolute risk reduction.** Turn 8. Absolute risk reduction is the difference between the two group risks, here ten per thousand or one percentage point.
- 5. percentage point.** Turn 10. A percentage point measures the difference between percentage values; four percent minus three percent equals one point.
- 6. relative risk reduction.** Turn 12. Relative risk reduction compares the decrease with the original comparison risk; ten divided by forty equals twenty-five percent.
- 7. Relative risk.** Turn 14. Relative risk is the intervention risk divided by the comparison risk, here thirty over forty or 0.75.
- 8. benefit framing.** Turn 16. Benefit framing is how a favorable effect is presented; balanced explanation includes absolute as well as relative figures.
- 9. population estimate.** Turn 18. A population estimate describes a group and does not guarantee the outcome for an individual person.
- 10. confidence interval.** Turn 20. A confidence interval expresses statistical uncertainty under defined assumptions and cannot be invented when no interval is supplied.

F Transfer conversation

Page 66

1. five

Clinician: "The absolute difference is five per thousand."

Twenty minus fifteen gives five events per thousand in this fictional comparison.

2. 0.5

Patient: "That is 0.5 percentage points."

Five divided by one thousand is half of one percent, or 0.5 percentage points.

3. twenty-five

Clinician: "The relative reduction is twenty-five percent."

Five divided by the comparison count of twenty equals twenty-five percent.

4. fictional

Patient: "These numbers remain fictional."

The figures are invented for practice and do not support a clinical recommendation.

08 Briefing and language checks

Coordinating rehabilitation and return to activity

A Briefing checks / page 67

1. What is confirmed?

A. Receipt of the rehabilitation referral

The receiving team has the referral, but later assessment and booking stages remain unresolved.

2. What barrier does Sam report?

A. Transport

Transport is the stated practical obstacle, not a lack of interest.

3. What requires individual assessment?

B. Exercise and work guidance

The exercise supplies no clearance or program from which to give activity instructions.

D Language checks / page 72

1. Which statement preserves the referral stage?

A. Received, but intake not yet confirmed.

The brief confirms receipt only and leaves intake and individual assessment unfinished.

2. Which question addresses access respectfully?

B. What makes travel to the sessions difficult?

Asking what makes travel difficult explores the stated barrier without inventing a negative attitude.

3. What must not be inferred from the referral?

A. A safe exercise intensity or unrestricted work clearance

Actual exercise and work guidance requires the appropriate individual clinical assessment.

4. Which promise is unsupported?

B. Every cost and journey will definitely be covered.

The case supplies no verified financial or transport entitlement to guarantee.

08 Conversation answers

Coordinating rehabilitation and return to activity

E Dialogue gaps

Pages 73-74

1. occupational demands. Turn 2. Occupational demands are the actual job tasks relevant to the work assessment.

2. return-to-work assessment. Turn 4. A return-to-work assessment evaluates actual readiness; rehabilitation attendance alone does not establish it.

3. Cardiac rehabilitation. Turn 6. Cardiac rehabilitation is supervised care, not automatic authorization for unrestricted activity.

4. intake confirmation. Turn 8. Intake confirmation establishes the receiving program's actual appointment, not merely referral receipt.

5. attendance barrier. Turn 10. An attendance barrier is a practical obstacle, not necessarily refusal of rehabilitation.

6. coverage verification. Turn 12. Coverage verification checks the actual financial arrangements and prevents an unsupported promise that every cost is covered.

7. exercise prescription. Turn 14. An exercise prescription specifies activity for an individual after appropriate assessment and should not be copied from another person's program.

8. rehabilitation goal. Turn 16. A rehabilitation goal is an individualized recovery or functioning objective that connects the program with the person's needs.

9. graded activity. Turn 18. Graded activity progresses in planned steps under an appropriate individual plan rather than an unsupplied universal schedule.

10. care coordinator. Turn 20. A care coordinator connects services and practical arrangements while clinical decisions remain with the appropriately responsible professionals.

F Transfer conversation

Page 75

1. lifting

Rory: "My job includes lifting."

Lifting is the actual work activity raised in the scenario.

2. demands

Clinician: "We need the task demands before assessing restrictions."

A job title alone does not specify the relevant physical requirements.

3. supplied

Rory: "No lifting limit has been supplied."

The exercise provides no numerical or clinical restriction to infer.

4. assessment

Clinician: "The work guidance needs individual assessment."

The appropriate clinical assessment must precede individualized work guidance.

08

09 Conversation answers

An alert from a cardiac device

E Extended conversation

Pages 76-77

- 1. device transmission.** Turn 4. A device transmission is information sent from the monitoring system and is not automatically a reviewed clinical finding.
- 2. alert.** Turn 6. An alert flags information for attention under the actual system and requires context rather than automatically establishing a diagnosis.
- 3. review workflow.** Turn 8. The review workflow describes how the service receives, assesses, and communicates device information rather than assuming every notification has already been interpreted.
- 4. urgent-care instructions.** Turn 12. Urgent-care instructions identify the appropriate clinical response to concerning symptoms and are not replaced by sending a routine device transmission.
- 5. Data availability.** Turn 16. Data availability concerns whether usable information reached the service and cannot itself establish the underlying clinical state when data are missing.
- 6. review status.** Turn 20. Review status identifies whether information is awaiting assessment or has been reviewed and should not be inferred solely from a receipt notification.

F Transfer exchange

Page 78

- 1. received**
Patient: "The transmission was received."
The service confirms that the device information arrived.
- 2. pending**
Clinician: "Clinical review remains pending."
Receipt does not mean the clinical interpretation is complete.
- 3. diagnosis**
Patient: "That is not a confirmed diagnosis."
A received transmission alone does not establish a clinical diagnosis.
- 4. status**
Clinician: "We should clarify the review status."
The relevant question is the actual stage of review and communication.

10 Conversation answers

Reconciling a cross-specialty medicine plan

E Extended conversation

Pages 79-80

1. document version. Turn 4. The document version identifies which dated instruction is being discussed and helps distinguish a superseded copy from a current plan.

2. procedural context. Turn 6. Procedural context includes the actual planned intervention and relevant circumstances, which must be known before medicine instructions are reconciled.

3. responsible prescriber. Turn 8. The responsible prescriber is the clinician accountable for the relevant medicine decision and should be identified explicitly when instructions conflict.

4. reconciled instruction. Turn 12. A reconciled instruction is the verified direction after conflicting information has been resolved by the appropriate clinicians.

5. communication ownership. Turn 16. Communication ownership assigns responsibility for explaining the verified plan and correcting the earlier conflicting message to the patient.

6. closed-loop reconciliation. Turn 20. Closed-loop reconciliation resolves the conflicting instructions and verifies that the appropriate recipients have the updated plan rather than merely documenting internal agreement.

F Transfer exchange

Page 81

1. verified

Clinician: "The current plan is verified."

The clinical plan has been checked and agreed by the relevant clinicians.

2. older

Office: "We are still sending the older letter."

The remaining problem is distribution of the superseded document.

3. correction

Clinician: "We need an authorized correction."

The outdated communication must be corrected through the proper record and document process.

4. communication

Office: "The patient also needs clear communication."

An internal correction alone does not ensure the patient receives the accurate plan.

11 Conversation answers

Discussing priorities in advanced heart disease

E Extended conversation

Pages 82-83

1. symptom burden. Turn 4. Symptom burden describes the impact of symptoms on daily life and helps connect clinical information with what matters to the patient.

2. Palliative care. Turn 6. Palliative care addresses symptoms, support, and quality of life and can be discussed alongside disease-directed care rather than automatically replacing it.

3. care goals. Turn 8. Care goals describe the outcomes and experiences important to the patient and are not automatically a specific resuscitation or treatment-withdrawal decision.

4. treatment burden. Turn 12. Treatment burden includes the practical and personal demands of care and should be considered with expected benefits and the patient's priorities.

5. advance care discussion. Turn 16. An advance care discussion explores preferences for future care and requires accurate documentation without claiming that every later circumstance or decision is already settled.

6. care coordinator. Turn 20. A care coordinator helps connect services and responsibilities, but the actual role and contact route must be confirmed rather than assumed.

F Transfer exchange

Page 84

1. priorities

Patient: "I want to revisit my priorities."

The patient asks to discuss a changed preference about the setting of care.

2. agreed

Clinician: "A treatment change is not yet agreed."

The scenario contains a request for discussion, not a completed decision.

3. heard

Patient: "My current wishes should be heard."

The patient's updated preferences need to inform the next discussion.

4. options

Clinician: "We will explain the actual options."

Available options require an appropriate clinical discussion rather than an assumed change.

11

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Giving an urgent cardiovascular handoff

Lead with the concern: I am handing over chest pressure with breathlessness.

Separate diagnosis: The cause is not established in this handoff.

Close the loop: I have your acknowledgment and the receiving contact.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Explaining ambulatory rhythm monitoring

Explain the purpose: We want to compare what you feel with the rhythm at that time.

Make the diary useful: Include the time, what you felt, and what you were doing.

Close the process: Let us confirm how the recording reaches the reviewing team.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Explaining an echocardiogram

Check the interpretation: What did that percentage suggest to you?

Name the chamber: This report refers to the left ventricle.

Separate a comparison: A change needs a comparable earlier study and clinical context.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Discussing atrial-fibrillation treatment goals

Separate the goals: Calming symptoms and preventing stroke are related but different goals.

Invite the concern: What worries you most about bleeding?

Agree on review: Let us review the relevant information before settling the plan.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Discussing coronary angiography consent

Check expectations: What have you understood will happen during the visit?

Clarify the agreement: We need to discuss exactly what you are consenting to.

Keep the decision open: No intervention decision is supplied by this conversation alone.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Checking a heart-failure home plan

Check the explanation: I want to check that I explained the two routes clearly.

Keep it individual: The thresholds and instructions must be the ones agreed for you.

Close with clarity: Please show me where the next review and contact details are recorded.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Explaining absolute and relative risk

State the denominator: Both figures are out of one thousand people.

Explain the difference: That is ten fewer events per thousand.

Keep uncertainty honest: No confidence interval has been supplied.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Coordinating rehabilitation and return to activity

Explain the service: Rehabilitation combines assessed activity, education, and support.

Describe the job: Tell us what the work requires physically.

Confirm the next step: Let us identify who will confirm the intake and contact you.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

absolute risk communicate absolute risk	59
absolute risk reduction explain absolute risk reduction	59
access-site complication explain access-site complications	42
action plan review the action plan	51
acute coronary syndrome evaluate suspected acute coronary syndrome	5
ambulatory monitor fit an ambulatory monitor	14
anticoagulation review anticoagulation	32
anticoagulation review arrange an anticoagulation review	33
antiplatelet therapy distinguish antiplatelet therapy	32
arrhythmia investigate a suspected arrhythmia	14
atrial fibrillation explain atrial fibrillation	32
atrial flutter distinguish atrial flutter	32
attendance barrier identify an attendance barrier	69
baseline risk explain baseline risk	59
benefit framing balance benefit framing	60
bleeding history clarify the bleeding history	33
bleeding risk assess bleeding risk	32
bradycardia assess bradycardia	14
cardiac catheterization discuss cardiac catheterization	41
cardiac rehabilitation explain cardiac rehabilitation	68
cardioversion explain cardioversion	32
care coordinator contact the care coordinator	69
care-plan discrepancy resolve a care-plan discrepancy	51
catheter ablation discuss catheter ablation	32

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

catheter laboratory attend the catheter laboratory	41
clinical handoff confirm the clinical handoff	6
clinical handoff confirm the clinical handoff	15
clinical handoff confirm the clinical handoff	24
clinical handoff confirm the clinical handoff	33
clinical handoff confirm the clinical handoff	42
clinical handoff confirm the clinical handoff	51
clinical handoff confirm the clinical handoff	60
clinical handoff confirm the clinical handoff	69
closed-loop escalation complete closed-loop escalation	6
comparison group identify the comparison group	59
comparison study review a comparison study	24
conduction evaluate cardiac conduction	15
confidence interval interpret a confidence interval	60
congestion assess congestion	50
contact pathway distinguish contact pathways	51
contrast medium explain contrast medium	41
coronary anatomy assess coronary anatomy	41
coronary angiography explain coronary angiography	41
coronary narrowing evaluate coronary narrowing	41
coverage verification obtain coverage verification	69
daily weight record review a daily weight record	50
decompensation assess suspected decompensation	51
denominator state the denominator	59

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

diagnostic procedure distinguish a diagnostic procedure	41
diaphoresis report associated diaphoresis	5
diastole describe diastolic filling	23
diastolic function assess diastolic function	24
diuretic explain a prescribed diuretic	50
documented plan review the documented plan	6
documented plan review the documented plan	15
documented plan review the documented plan	24
documented plan review the documented plan	33
documented plan review the documented plan	42
documented plan review the documented plan	51
documented plan review the documented plan	60
documented plan review the documented plan	69
drug interaction check for a drug interaction	33
dyspnea describe reported dyspnea	5
echocardiogram explain an echocardiogram	23
ectopic beat describe an ectopic beat	14
edema describe peripheral edema	50
ejection fraction explain left-ventricular ejection fraction	23
electrocardiogram interpret an electrocardiogram	14
electrode lead check an electrode lead	15
emergency activation confirm emergency activation	6
event marker use the event marker	14
event rate compare event rates	59

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

exercise prescription	review an exercise prescription	68
exercise tolerance	describe exercise tolerance	50
exertional symptom	clarify exertional symptoms	5
femoral access	discuss femoral access	41
fluid balance	assess fluid balance	50
fluid restriction	clarify a prescribed fluid restriction	50
follow-up interval	confirm the follow-up interval	51
functional capacity	assess functional capacity	68
graded activity	discuss graded activity	68
guideline-directed therapy	review guideline-directed therapy	50
heart failure	explain heart failure	50
hemodynamic status	communicate hemodynamic status	5
image quality	describe image quality	24
individual risk estimate	explain an individual risk estimate	33
initial assessment	report the initial assessment	5
intake confirmation	obtain intake confirmation	69
intervention group	describe the intervention group	59
left atrial appendage	explain the left atrial appendage	32
left ventricle	assess the left ventricle	23
monitor return	confirm monitor return	15
myocardial infarction	confirm myocardial infarction	5
myocardial ischemia	assess myocardial ischemia	5
natural frequency	use natural frequencies	59
number needed to treat	qualify number needed to treat	60

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

occupational demand describe occupational demands	68
orthopnea ask about orthopnea	50
palpitations describe palpitations	14
paroxysmal describe paroxysmal symptoms	14
paroxysmal nocturnal dyspnea clarify nocturnal dyspnea	50
patient preference elicit the patient preference	6
patient preference elicit the patient preference	15
patient preference elicit the patient preference	24
patient preference elicit the patient preference	33
patient preference elicit the patient preference	42
patient preference elicit the patient preference	51
patient preference elicit the patient preference	60
patient preference elicit the patient preference	69
percentage point distinguish percentage points	59
percutaneous coronary intervention discuss percutaneous coronary intervention	41
population estimate interpret a population estimate	60
postprocedure observation explain postprocedure observation	42
prehospital report relay the prehospital report	6
preparation instruction verify preparation instructions	42
preserved ejection fraction discuss preserved ejection fraction	23
pressure estimate qualify a pressure estimate	24
procedural risk discuss procedural risk	42
procedure consent confirm procedure consent	42
program eligibility verify program eligibility	69

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

qualified interpreter	request a qualified interpreter	6
qualified interpreter	request a qualified interpreter	15
qualified interpreter	request a qualified interpreter	24
qualified interpreter	request a qualified interpreter	33
qualified interpreter	request a qualified interpreter	42
qualified interpreter	request a qualified interpreter	51
qualified interpreter	request a qualified interpreter	60
qualified interpreter	request a qualified interpreter	69
radial access	discuss radial access	41
rate control	discuss rate control	32
receiving team	identify the receiving team	6
recording period	confirm the recording period	14
reduced ejection fraction	explain reduced ejection fraction	23
rehabilitation goal	set a rehabilitation goal	68
rehabilitation intake	arrange rehabilitation intake	68
relative risk	calculate relative risk	59
relative risk reduction	explain relative risk reduction	59
renal function	review renal function	33
rest symptom	report symptoms at rest	5
retrosternal	describe retrosternal discomfort	5
return-to-work assessment	request a return-to-work assessment	68
rhythm control	discuss rhythm control	32
right ventricle	assess the right ventricle	23
risk communication	improve risk communication	60

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

risk-factor modification	support risk-factor modification	68
secondary prevention	discuss secondary prevention	68
sedation	explain the sedation plan	42
self-monitoring	explain self-monitoring	51
serial testing	explain serial testing	5
shared treatment decision	reach a shared treatment decision	33
signal artifact	identify signal artifact	15
signal quality	assess signal quality	15
sinus rhythm	identify sinus rhythm	14
sodium intake	discuss sodium intake	50
source attribution	preserve source attribution	6
source attribution	preserve source attribution	15
source attribution	preserve source attribution	24
source attribution	preserve source attribution	33
source attribution	preserve source attribution	42
source attribution	preserve source attribution	51
source attribution	preserve source attribution	60
source attribution	preserve source attribution	69
stent	explain a coronary stent	41
stroke risk	assess stroke risk	32
stroke volume	distinguish stroke volume	23
structural heart disease	investigate structural heart disease	24
supervised exercise	arrange supervised exercise	68
symptom diary	keep a symptom diary	14

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

symptom trajectory describe the symptom trajectory	6
symptom-rhythm correlation assess symptom-rhythm correlation	14
systole describe systolic function	23
tachycardia assess tachycardia	15
teach-back use teach-back without blame	6
teach-back use teach-back without blame	15
teach-back use teach-back without blame	24
teach-back use teach-back without blame	33
teach-back use teach-back without blame	42
teach-back use teach-back without blame	51
teach-back use teach-back without blame	60
teach-back use teach-back without blame	69
thromboembolism discuss thromboembolism	32
time horizon specify the time horizon	59
time-critical concern communicate a time-critical concern	6
transport arrangement confirm transport arrangements	69
troponin interpret a troponin result	5
uncertainty range explain the uncertainty range	60
valvular regurgitation assess valvular regurgitation	23
valvular stenosis explain valvular stenosis	23
vascular access explain vascular access	41
ventricular volume measure ventricular volume	24
wall motion assess regional wall motion	23
work restriction clarify work restrictions	68

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] American Heart Association. Ejection Fraction Heart Failure Measurement.

[Open the original source](#)

Background for explaining the measurement and why it is only part of a clinical assessment. Checked 10 October 2026.

[2] American Heart Association. Atrial Fibrillation Medications.

[Open the original source](#)

Context for distinct treatment goals and stroke/bleeding discussions, not a prescribing guide. Checked 10 October 2026.

[3] American Heart Association. Coronary Angiogram.

[Open the original source](#)

Background for explaining diagnostic coronary angiography and related terminology. Checked 10 October 2026.

[4] American Heart Association. Cardiac Rehabilitation for Heart Failure.

[Open the original source](#)

Context for medically supervised rehabilitation and practical access questions. Checked 10 October 2026.

Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.

No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.

All people, records, figures, and encounters are fictional. This is professional English practice, not medical advice, clinical training, a treatment protocol, or certification. Use current evidence, authorized supervision where required, local law, and your actual clinical and emergency procedures. Do not use an exercise to decide that symptoms can safely wait, select treatment, or calculate a dose. Communication about privacy, consent, and professional duties depends on jurisdiction and setting. Use qualified language support and protect real patient information.

English Ladder | English for Work | 2026 edition. The B2-C1 range is an editorial study recommendation, not a proficiency certification.

[Open the matching English Ladder course](#)